

ENGLISH FOR GENERAL PRACTITIONERS

General Practice English

Hear the concern. Make the plan clear.

Phil Stilwell

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Eight lessons. Eleven extended conversations.

INSIDE THE BOOK

192 vocabulary entries / 128 reusable phrases
220 extended-dialogue turns / 11 transfer exchanges
Word banks and explained answer keys

Upper-intermediate to advanced (B2-C1). For general practitioners, family physicians, and primary-care doctors.

2026 edition

Eight scenarios. Eight skills.

Eight consultations move from a shared agenda to accurate histories, medicines, test results, antibiotic expectations, screening, mental health, and referral follow-up.

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Clinical precision. Everyday language.

Primary care combines continuity with new concerns. These conversations practice how a doctor listens, explains the limits of evidence, negotiates a plan, and keeps responsibility visible.

Start with the patient agenda

Elicit the concern behind the appointment label. Explain how you will prioritize competing needs without treating a new concern as an interruption.

"Which concern most needs our attention today?"

Keep the timeline intact

A symptom account has a source, timing, and context. Ask what changed before compressing the history into clinical shorthand.

"The tiredness began before the medicine was started."

Explain uncertainty usefully

Name what is established, what remains possible, and the next step. Reassurance should not become an unsupported promise.

"This result needs context; it does not establish a diagnosis by itself."

Close the communication loop

A requested test or referral creates follow-up work. Identify who reviews it, how the patient hears back, and how unresolved concerns are escalated.

"Our team will confirm receipt; that is different from a booked appointment."

PROFESSIONAL BOUNDARIES

All people, records, figures, and encounters are fictional. This is professional English practice, not medical advice, clinical training, a treatment protocol, or certification. Use current evidence, authorized supervision where required, local law, and your actual clinical and emergency procedures. Do not use an exercise to decide that symptoms can safely wait, select treatment, or calculate a dose. Communication about privacy, consent, and professional duties depends on jurisdiction and setting. Use qualified language support and protect real patient information.

Three concerns, one visit

A Read the situation

Negotiate priorities without dismissing a newly disclosed concern.

Dr Shah sees Morgan for a planned long-term-condition review. Morgan also wants to discuss tiredness and a form for work. No clinical urgency assessment has yet been made. Morgan says the tiredness matters most. Dr Shah will first clarify that concern and assess priorities, then explain what can be covered and what requires follow-up. No symptom is declared safe to postpone merely because the appointment was booked for another purpose.

PEOPLE / Dr Shah: General practitioner | Morgan: Patient

AN AGENDA IS NEGOTIATED, NOT IMPOSED

Invite the person to name their concerns before announcing the visit structure. A follow-up arrangement is a specific agreement, not a way to quietly drop an unresolved concern.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What was originally planned?

- A A confirmed emergency discharge
- C A long-term-condition review

- B A completed tiredness investigation
- D A work-capacity assessment only

2. Which concern does Morgan prioritize?

- A A referral already completed
- C The work form

- B Tiredness
- D A medicine already prescribed today

3. What has not yet happened?

- A The visit being booked
- C Clinical assessment of urgency

- B Morgan naming a priority
- D Morgan mentioning the form

Answers and explanations: [page 85](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

consultation agenda

Concerns and tasks agreed for a clinical encounter.

IN USE agree on the consultation agenda

presenting concern

Problem the patient brings to the current encounter.

IN USE elicit the presenting concern

continuity of care

Connected care over time with an ongoing clinical relationship.

IN USE maintain continuity of care

comorbidity

Another condition occurring alongside a particular condition.

IN USE review relevant comorbidity

multimorbidity

Presence of multiple long-term health conditions in one person.

IN USE manage multimorbidity

problem list

Organized record of a patient's identified health problems.

IN USE update the problem list

acute concern

A new or recently changing health concern.

IN USE assess an acute concern

long-term condition

Health condition requiring ongoing management or follow-up.

IN USE review a long-term condition

patient-centered care

Care attentive to the person's needs, priorities, and circumstances.

IN USE provide patient-centered care

functional limitation

Restriction affecting a person's usual activities or abilities.

IN USE describe a functional limitation

follow-up visit

Later encounter to review progress or unfinished care.

IN USE arrange a follow-up visit

priority

Issue judged to require attention before other issues.

IN USE establish the clinical priority

Precision check

A booked review does not determine the urgency of a newly raised symptom. Agree on the agenda after appropriate clinical assessment, not before it.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

shared agenda

Visit priorities agreed through discussion with the patient.

IN USE negotiate a shared agenda

visit length

Time allocated or used for an encounter.

IN USE explain the visit length

care coordination

Organizing related care across people, services, or settings.

IN USE improve care coordination

concern elicitation

Inviting the patient to describe what needs attention.

IN USE use open concern elicitation

signposting

Explaining where a conversation is going next.

IN USE use clear signposting

safety-netting

Explaining what changes require further help and how to obtain it.

IN USE provide specific safety-netting

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

A work form, a symptom, and a chronic-condition review are different tasks. Summarize all three so that prioritizing one does not erase the others.

Phrases you can actually use

C Notice the language / 1 of 2

OPEN THE AGENDA

"What else were you hoping we could cover?"

EXPLAIN SEQUENCING

"I will clarify that concern before we agree on the order."

KEEP ANOTHER TASK VISIBLE

"The work form is still on our list."

USE A TRANSITION

"First I will ask about the tiredness; then we will review the plan."

REFLECT THE PRIORITY

"The tiredness is the main concern for you today."

AVOID DISMISSAL

"I am not setting it aside because it was not on the booking."

NEGOTIATE FOLLOW-UP

"Let us agree how to handle anything we cannot complete today."

CHECK PRIORITIES

"Have I captured the concerns in the right order?"

What else

Invites additional concerns without implying that the patient has used up permission to speak.

Before we agree

Makes the order conditional on clarification and assessment.

Still on our list

Reassures the patient that a deferred task has not been forgotten.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which response avoids dismissing the symptom?

- A** That was not booked, so it cannot matter today.
- B** The form proves you are well.
- C** A review slot makes tiredness nonurgent.
- D** Let us clarify the tiredness before deciding the agenda.

2. Which wording preserves the patient's preference?

- A** Morgan refuses all chronic care.
- B** Morgan has only an administrative concern.
- C** Morgan requests a diagnosis already confirmed.
- D** Morgan identifies tiredness as the main concern.

3. What does follow-up mean here?

- A** Permission to ignore new symptoms
- B** A guarantee that no assessment is needed
- C** Automatic completion of the form
- D** An agreed later review of unfinished work

4. Which signpost is clearest?

- A** Nothing new can be raised.
- B** First the tiredness history, then we will agree on the remaining plan.
- C** We will get to whatever matters eventually.
- D** Everything is sorted already.

Answers and explanations: [page 85](#).

Three concerns, one visit

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

signposting / priority / consultation agenda / functional limitation / shared agenda / safety-netting / continuity of care / long-term condition / presenting concern / follow-up visit

- 01 **Dr Shah:** We booked a review of your ongoing condition. Before we start, what else were you hoping to discuss?
- 02 **Morgan:** I have been unusually tired, and I brought a work form. The tiredness is my main ¹ _____.
- 03 **Dr Shah:** Thank you. I will ask about the tiredness before we decide which issues need attention first.
- 04 **Morgan:** I thought the ² _____ was fixed because the reminder only mentioned a review.
- 05 **Dr Shah:** The booking helps us prepare, but it does not replace listening to a new concern or assessing it.
- 06 **Morgan:** That helps. I did not want the form to make this look like an entirely administrative ³ _____.
- 07 **Dr Shah:** Let us keep all three items visible. Tell me first when the tiredness began and how it affects you.
- 08 **Morgan:** I am struggling with ordinary tasks. I suppose that is a ⁴ _____.
- 09 **Dr Shah:** Exactly. Describe those tasks in your own words; I do not need you to use a clinical label.
- 10 **Morgan:** Can we still review the ⁵ _____ rather than losing that appointment altogether?

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 10.

- 11 **Dr Shah:** Yes, we will account for it in the plan. First I need enough information to assess today's priorities.
- 12 **Morgan:** I appreciate that ⁶ _____. I understand why you are asking about the tiredness now.
- 13 **Dr Shah:** After that assessment, we can agree what to complete today and how to address anything left unfinished.
- 14 **Morgan:** A specific ⁷ _____ would help more than being told to call sometime.
- 15 **Dr Shah:** We will make the purpose clear and explain how to obtain help if the situation changes before then.
- 16 **Morgan:** So ⁸ _____ is part of the plan, not a promise that nothing will happen.
- 17 **Dr Shah:** Correct. We also need to clarify the form's requirements before promising what I can certify.
- 18 **Morgan:** Then our ⁹ _____ begins with the tiredness, while the review and form remain visible.
- 19 **Dr Shah:** That captures your preference. I will assess the concern before we settle the clinical order and next steps.
- 20 **Morgan:** Good. I want ¹⁰ _____, not three disconnected conversations in which the details get lost.

ANSWERS / EXPLANATIONS: PAGE 86

1. presenting concern | 2. consultation agenda | 3. priority | 4. functional limitation | 5. long-term condition | 6. signposting | 7. follow-up visit | 8. safety-netting | 9. shared agenda | 10. continuity of care

Keep a second concern visible

F Say it

Patient Lee attends a blood-pressure review and raises a new sleep concern. Dr Chen will clarify the sleep concern before agreeing on priorities. A travel form remains unfinished.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

sleep / unfinished / review / clarify

Dr Chen: "The planned visit is a blood-pressure ¹ _____."

Lee: "I also want to discuss my ² _____."

Dr Chen: "I will first ³ _____ that concern."

Lee: "The travel form remains ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 86

1. review | 2. sleep | 3. clarify | 4. unfinished

Rehearse with a fixed script

1. Read the corrected agenda exchange. Stress tiredness, review, and work form as three separate concerns.
2. Swap roles. Repeat the signposting lines and the distinction between a proposed agenda and assessed priorities.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Dizziness means different things

A Read the situation

Clarify a patient word and reconstruct a timeline without supplying the answer.

Ari uses the word dizzy for episodes that began on Tuesday. Ari describes feeling about to faint, not the room spinning. Episodes are reported after standing, and their duration is not yet clear. Dr Cole has not established a cause. The task is to clarify the account and record relevant unknowns, not diagnose from one trigger or assume unmentioned symptoms are absent.

PEOPLE / Dr Cole: General practitioner | Ari: Patient

LET THE DESCRIPTION PRECEDE THE LABEL

A familiar word may mean different sensations to different people. Offer neutral clarification and preserve the person's wording before using professional shorthand.

Briefing checks

Circle one answer in each item. Use the briefing.

1. When did the episodes begin according to Ari?

- A Before any date can be named
- C During a completed admission

- B After a confirmed treatment change
- D Tuesday

2. What sensation does Ari describe?

- A No symptoms at all
- C The room definitely spinning

- B Feeling about to faint
- D A confirmed seizure

3. What remains unclear?

- A Whether Dr Cole has met Ari
- C Whether Ari used the word dizzy

- B Episode duration
- D Whether standing was mentioned

Answers and explanations: page 87.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

onset

Time at which a symptom first began.

IN USE establish the reported onset

duration

Length of time a symptom or episode lasts.

IN USE clarify episode duration

frequency

How often a symptom or event occurs.

IN USE describe symptom frequency

trigger

Event or circumstance associated with starting a symptom.

IN USE identify a reported trigger

relieving factor

Circumstance associated with reducing a symptom.

IN USE ask about relieving factors

associated symptom

Another symptom occurring alongside the main symptom.

IN USE ask about associated symptoms

pertinent negative

Relevant absence established through questioning or assessment.

IN USE document a pertinent negative

progression

Change in a symptom or condition over time.

IN USE describe symptom progression

episodic

Occurring in separate periods rather than continuously.

IN USE describe episodic symptoms

persistent

Continuing rather than resolving between distinct episodes.

IN USE report persistent discomfort

location

Place in the body where a symptom is experienced.

IN USE specify the symptom location

radiation

Spread of a symptom such as pain to another area.

IN USE describe pain radiation

Precision check

A reported trigger is not a demonstrated cause. Dizziness, vertigo, and feeling faint should not be substituted for one another without clarification.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

character

Quality of a symptom, such as pressure or burning.

IN USE clarify symptom character

severity

Degree or intensity of a symptom or condition.

IN USE assess symptom severity

chronology

Order in which events occurred over time.

IN USE preserve the chronology

review of systems

Structured questioning about symptoms across body systems.

IN USE conduct a focused review of systems

history of present illness

Organized account of the current clinical problem.

IN USE summarize the history of present illness

diagnostic uncertainty

Lack of a sufficiently established explanation for the condition.

IN USE communicate diagnostic uncertainty

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Not asked, not reported, and denied are different. Record an unanswered duration question as unanswered, not as a normal or reassuring duration.

Phrases you can actually use

C Notice the language / 1 of 2

CLARIFY A LABEL

"What does dizzy feel like for you?"

OFFER A NEUTRAL DISTINCTION

"Is it spinning, feeling faint, or something different?"

LOCATE THE ONSET

"When did you first notice it?"

SEPARATE TWO TIMES

"Is that the length of one episode or the time since it began?"

AVOID A SUPPLIED ANSWER

"Describe what happens next in your own words."

KEEP A GAP VISIBLE

"We have not yet clarified how long each episode lasts."

ATTRIBUTE THE TRIGGER

"You report noticing it after standing."

SUMMARIZE CAREFULLY

"I have the sequence; I have not established the cause."

Since versus for

Since introduces a starting point; for introduces a duration. Since Tuesday does not tell us how long one episode lasts.

When versus why

A when question asks about timing; it does not establish why the symptom happens.

Not yet clarified

Names missing information without converting it into an absent symptom.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

MAKE SPACE

"We can pause before moving to the next point."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

EXPLAIN A TERM

"In everyday language, that means the following."

CHECK YOUR EXPLANATION

"Let me explain that another way."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which sentence distinguishes onset from duration?

- A** Tuesday proves a cause.
- B** Duration has been measured already.
- C** Episodes began Tuesday; the length of each is still unclear.
- D** Each episode has lasted continuously since Tuesday.

2. Which phrase overstates the evidence?

- A** Ari reports symptoms after standing.
- B** The cause is not established.
- C** Ari describes feeling faint.
- D** Standing definitely caused the condition.

3. Which question is least leading?

- A** You agree this is harmless, yes?
- B** What happens during an episode?
- C** It is just stress, correct?
- D** You never have any other symptoms, right?

4. What is a pertinent negative?

- A** A clinician's guess about what is unlikely
- B** A relevant absence established by asking or assessing
- C** Any symptom not mentioned spontaneously
- D** A blank record field

Answers and explanations: [page 87](#).

Dizziness means different things

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

duration / associated symptom / diagnostic uncertainty / trigger / pertinent negative / onset / relieving factor / chronology / episodic / frequency

- 01 **Dr Cole:** You used the word dizzy on the appointment request. What does that feel like for you?
- 02 **Ari:** More like I might faint than like the room is spinning. The ¹ _____ was Tuesday.
- 03 **Dr Cole:** Thank you. Did you feel it continuously from Tuesday, or have there been separate episodes?
- 04 **Ari:** Separate episodes. I suppose ² _____ describes it, but I cannot time them accurately yet.
- 05 **Dr Cole:** That is useful to know. How often have you noticed it, and what are you doing when it starts?
- 06 **Ari:** Standing up seems to be a ³ _____, although I have not kept a record of every episode.
- 07 **Dr Cole:** I will record that as your observation. What happens next, and what makes the feeling ease?
- 08 **Ari:** Sitting down seems to help. That is the main ⁴ _____ I have noticed.
- 09 **Dr Cole:** When you say it lasts a while, do you mean seconds, minutes, or something you cannot estimate?
- 10 **Ari:** I cannot estimate the ⁵ _____. I do not want to give you a number just to complete the question.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 19.

- 11 **Dr Cole:** Please do not guess. I will ask about other symptoms separately rather than treating silence as an answer.
- 12 **Ari:** So an ⁶ _____ is something else happening during the same episode?
- 13 **Dr Cole:** Yes. I also need to understand any changes and how the episodes affect what you can do.
- 14 **Ari:** The ⁷ _____ may be increasing, but I should explain what I actually remember.
- 15 **Dr Cole:** Exactly. Start with Tuesday and describe the later events in order, including anything that is uncertain.
- 16 **Ari:** That will keep the ⁸ _____ straight instead of mixing the first episode with the most recent one.
- 17 **Dr Cole:** I will summarize your account, then assess it. The standing association alone does not establish a diagnosis.
- 18 **Ari:** I understand the ⁹ _____. You are listening to the pattern, not promising an explanation yet.
- 19 **Dr Cole:** Correct. Any relevant absence I record will come from a question or assessment, not from an empty field.
- 20 **Ari:** Then a ¹⁰ _____ is different from something I simply have not mentioned.

ANSWERS / EXPLANATIONS: PAGE 88

1. onset | 2. episodic | 3. trigger | 4. relieving factor | 5. duration | 6. associated symptom | 7. frequency | 8. chronology | 9. diagnostic uncertainty | 10. pertinent negative

Separate the first day from episode length

F Say it

Jamie reports intermittent tingling beginning Friday. Individual episodes last about two minutes by Jamie's estimate. Dr Park has not established a cause.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

intermittent / Friday / established / two

Dr Park: "The reported onset was ¹ _____."

Jamie: "One episode lasts about ² _____ minutes."

Dr Park: "The symptoms are ³ _____."

Dr Park: "The cause is not yet ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 88

1. Friday | 2. two | 3. intermittent | 4. established

Rehearse with a fixed script

1. Read Ari's corrected symptom history. Contrast onset, duration, and frequency with clear stress.
2. Swap roles. Repeat the neutral clarification question and the summary that preserves diagnostic uncertainty.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Two lists and an unfamiliar packet

A Read the situation

03

Investigate a medication discrepancy while separating actual use from recorded instructions.

Robin brings a printed hospital list and a packet bought without a prescription. The clinic list differs from the hospital list. Dr Lin has not verified which instructions are current or whether products share an ingredient. Robin sometimes misses a medicine because of cost. No dose changes are supplied. The conversation must establish actual use, verify the conflicting records, and address the cost barrier without calling Robin noncompliant.

PEOPLE / Dr Lin: General practitioner | Robin: Patient

ASK WHAT HAPPENS, NOT WHETHER SOMEONE OBEYS

A list records an instruction or history; it does not prove what the person takes. Ask about practical barriers in neutral language.

Briefing checks

Circle one answer in each item. Use the briefing.

1. Which records differ?

- | | |
|---|---|
| A The clinic and hospital lists | B Two verified identical prescriptions |
| C A completed laboratory report and scan | D No records differ |

2. What barrier does Robin report?

- | | |
|--|--------------------------------------|
| A An instruction to stop every medicine | B A completed dose adjustment |
| C Cost | D A proven drug allergy |

3. What has not been verified?

- | | |
|---|--|
| A Whether there is a printed list | B Whether the clinic has a record |
| C Whether the products share an ingredient | D Whether Robin brought a packet |

Answers and explanations: [page 89](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

medication reconciliation

Comparing medicine information to resolve discrepancies across records and actual use.

IN USE perform medication reconciliation

medicine list

Record of medicines and their relevant instructions.

IN USE verify the medicine list

over-the-counter product

Product available without a prescription under local rules.

IN USE disclose over-the-counter products

active ingredient

Substance responsible for a medicine's intended pharmacological effect.

IN USE compare active ingredients

brand name

Commercial name used to market a product.

IN USE record the brand name

generic name

Standard nonproprietary name of a medicinal substance.

IN USE confirm the generic name

duplicate therapy

Unintended or unnecessary overlap between therapeutic products or treatments.

IN USE check for duplicate therapy

adherence

Extent to which actual medicine use matches an agreed plan.

IN USE discuss barriers to adherence

adverse reaction

Unwanted harmful response associated with a medicinal product.

IN USE describe an adverse reaction

drug allergy

Immune-mediated reaction to a medicine.

IN USE clarify a reported drug allergy

intolerance

Difficulty tolerating a medicine that is not necessarily an allergy.

IN USE distinguish intolerance from allergy

interaction

Effect of one substance or treatment on another.

IN USE assess a possible interaction

Precision check

Do not treat a different brand name as proof of a different ingredient. Verify products and instructions before resolving the clinical question.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

dosage form

Physical preparation in which a medicine is supplied.

IN USE verify the dosage form

route of administration

Path by which a medicine enters the body.

IN USE confirm the route of administration

discontinued medicine

Medicine recorded as no longer intended for use.

IN USE verify a discontinued medicine

record discrepancy

Difference between records requiring investigation or clarification.

IN USE resolve a record discrepancy

prescriber

Authorized professional issuing a medicine prescription.

IN USE contact the original prescriber

medication review

Structured evaluation of medicine use and related needs.

IN USE arrange a medication review

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

An adverse reaction, an intolerance, and an allergy are not interchangeable labels. Preserve the reported reaction and obtain the relevant clinical history.

Phrases you can actually use

C Notice the language / 1 of 2

03

ASK ABOUT ACTUAL USE

"Walk me through what you actually take."

NAME THE DISCREPANCY

"These two lists do not match."

ASK ABOUT AFFORDABILITY

"Has cost made the agreed plan difficult to follow?"

SEPARATE NAMES

"Different brand names can still contain the same ingredient."

NORMALIZE DISCLOSURE

"Please include products bought without a prescription."

AVOID PREMATURE CORRECTION

"I need to verify the current instructions first."

CLARIFY A REACTION

"What happened when you took it?"

AGREE ON VERIFICATION

"We will check the source before finalizing the list."

Actually take

Distinguishes real use from instructions recorded elsewhere.

What happened

Elicits the reaction instead of confirming an assumed allergy.

Before finalizing

Identifies a verification step that must precede a settled record.

Phrases you can actually use

C Notice the language / 2 of 2

03

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which question avoids blame?

- A** Why do you ignore doctors?
- B** Why are you always noncompliant?
- C** You chose to waste the medicine, correct?
- D** Has cost made the plan difficult to follow?

2. Which conclusion is unsupported?

- A** The lists differ.
- B** The packet needs checking.
- C** Robin reports a cost barrier.
- D** Different brands prove different ingredients.

3. Which distinction matters?

- A** A prescription versus proof of adherence
- B** Actual use versus recorded instructions
- C** Packet color versus appointment time only
- D** Guess versus another guess

4. What should happen before finalizing the list?

- A** Verification of conflicting information
- B** Automatic deletion of the hospital list
- C** An invented dose change
- D** Recording every reaction as an allergy

Answers and explanations: [page 89](#).

Two lists and an unfamiliar packet

E Conversations / turns 1-10

03

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

active ingredient / medicine list / intolerance / medication review / over-the-counter product / adherence / drug allergy / medication reconciliation / brand name / record discrepancy

- 01 **Dr Lin:** Please show me both lists, and include anything you buy yourself or take only occasionally.
- 02 **Robin:** I brought this ¹ _____. I did not think it belonged with my prescriptions.
- 03 **Dr Lin:** It matters too. First, tell me what you actually take rather than reading only the printed instructions.
- 04 **Robin:** My hospital ² _____ differs from the clinic version, so I am uncertain which is current.
- 05 **Dr Lin:** We need to verify that difference. I will not silently choose one record because it looks newer.
- 06 **Robin:** Is this what you mean by ³ _____ rather than simply adding another packet to the computer?
- 07 **Dr Lin:** Yes. We will check the products, instructions, and sources, and establish which questions remain unresolved.
- 08 **Robin:** This packet has a different ⁴ _____. Does that mean it cannot overlap with another medicine?
- 09 **Dr Lin:** Not necessarily. We need the ingredient information before drawing that conclusion or making a clinical decision.
- 10 **Robin:** Then we should compare the ⁵ _____, not rely on the appearance of the packaging.

The conversation, continued

E Conversations / turns 11-20

03

Continue with the word bank on page 28.

- 11 **Dr Lin:** Correct. Also, how manageable has the agreed medicine plan been in everyday life?
- 12 **Robin:** Cost has affected my ⁶ _____. Sometimes I cannot afford to collect a medicine.
- 13 **Dr Lin:** Thank you for telling me. That is important information for planning care, not a reason to blame you.
- 14 **Robin:** I also described one medicine as an allergy, but I am unsure whether it was ⁷ _____.
- 15 **Dr Lin:** Tell me exactly what happened and when. We should preserve the reaction details before settling the label.
- 16 **Robin:** I understand. A recorded ⁸ _____ should not be invented from an incomplete description.
- 17 **Dr Lin:** I will clarify the clinical history and contact the relevant source about the conflicting instructions.
- 18 **Robin:** Please make the ⁹ _____ visible until it is resolved, so another person does not assume it was settled.
- 19 **Dr Lin:** Agreed. We have not decided any dose change in this discussion; that requires the appropriate clinical review.
- 20 **Robin:** Then today's ¹⁰ _____ will include affordability as well as an accurate list.

ANSWERS / EXPLANATIONS: PAGE 90

1. over-the-counter product | 2. medicine list | 3. medication reconciliation | 4. brand name | 5. active ingredient
| 6. adherence | 7. intolerance | 8. drug allergy | 9. record discrepancy | 10. medication review

Verify an old entry

F Say it

Taylor says an outside prescriber stopped one medicine, but the local list still includes it. Dr Noor will verify the instruction. The date is unknown; no new dose has been authorized.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

unknown / verify / prescriber / includes

Taylor: "An outside ¹ _____ gave the instruction."

Dr Noor: "The local list still ² _____ the medicine."

Taylor: "The instruction date is ³ _____."

Dr Noor: "I will ⁴ _____ the instruction."

ANSWERS / EXPLANATIONS: PAGE 90

1. prescriber | 2. includes | 3. unknown | 4. verify

Rehearse with a fixed script

1. Read the corrected review. Stress actually take, recorded instructions, and verify as distinct ideas.
2. Swap roles. Repeat the affordability question and the reaction-history question without adding blame or a dose change.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Outside the range is not the diagnosis

A Read the situation

04

Explain a result without equating a laboratory flag with a confirmed condition.

Casey sees an out-of-range flag on a blood-test report before speaking with Dr Vega. No numerical value or diagnosis is supplied in this case. Dr Vega will review the result alongside symptoms, previous results, and sample context. A repeat test is being considered, not yet ordered. Casey fears that the flag means a serious diagnosis is certain.

PEOPLE / Dr Vega: General practitioner | Casey: Patient

EXPLAIN THE FLAG BEFORE OFFERING REASSURANCE

A patient may encounter technical results before receiving an explanation. Acknowledge the uncertainty and explain the review process without promising that a flagged result is harmless.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What has Casey seen?

- A A completed repeat test
- C An out-of-range flag

- B A treatment response
- D A confirmed diagnosis supplied in this case

2. What is the repeat-test status?

- A Already performed
- C Definitely unnecessary

- B Already normal
- D Under consideration

3. What will Dr Vega use alongside the result?

- A The flag color alone
- C A diagnosis invented for the exercise

- B A guess from the patient portal
- D Symptoms, previous results, and sample context

Answers and explanations: [page 91](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

laboratory result

Reported outcome of a test on a specimen.

IN USE interpret a laboratory result

reference interval

Range used by a laboratory for comparison with a result.

IN USE explain the reference interval

out-of-range flag

Indicator that a result lies outside a stated comparison range.

IN USE review an out-of-range flag

trend

Pattern of change across comparable observations over time.

IN USE examine the result trend

baseline

Earlier or usual state used for comparison.

IN USE compare with the baseline

preanalytical variation

Differences arising before a sample is analyzed.

IN USE review preanalytical variation

false-negative result

Negative test result when the target condition is actually present.

IN USE explain a false-negative result

false-positive result

Positive test result when the target condition is actually absent.

IN USE explain a false-positive result

repeat sample

New specimen obtained to repeat an investigation.

IN USE request a repeat sample

incidental finding

Finding discovered outside the main purpose of an investigation.

IN USE explain an incidental finding

clinical significance

Meaning or importance of a finding for patient care.

IN USE assess clinical significance

differential diagnosis

Set of possible explanations being evaluated clinically.

IN USE discuss the differential diagnosis

Precision check

Outside a reference interval is not a diagnosis. Normal, negative, and no further action are also different statements, each requiring context.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

working diagnosis

Provisional explanation guiding assessment or care.

IN USE qualify a working diagnosis

confirmatory test

Investigation used to help establish a suspected finding.

IN USE discuss a confirmatory test

sensitivity

Ability of a test to identify people with the target condition.

IN USE explain test sensitivity

specificity

Ability of a test to identify people without the target condition.

IN USE explain test specificity

predictive value

How strongly a result predicts the target condition in its context.

IN USE discuss predictive value

pending result

Test outcome that has not yet become available.

IN USE track a pending result

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

A possible repeat is not an order, and an order is not a completed result. Keep the stage of the investigation explicit.

Phrases you can actually use

C Notice the language / 1 of 2

ACKNOWLEDGE THE WORRY

"Seeing a flag before an explanation can be unsettling."

LIMIT THE CONCLUSION

"The flag alone does not establish a diagnosis."

STATE A PROPOSAL

"A repeat is one option under consideration."

CLARIFY STATUS

"No repeat has been ordered yet."

EXPLAIN THE MARKER

"The flag shows how the result compares with the laboratory range."

BRING IN CONTEXT

"I will compare it with your symptoms and previous results."

AVOID FALSE REASSURANCE

"I cannot call it harmless before reviewing the context."

CHECK THE MEANING

"What does the flag mean to you after that explanation?"

Does not establish

Limits a conclusion without claiming that the opposite has been proved.

Under consideration

Describes a proposal rather than a completed clinical action.

Compared with

Introduces the reference used to interpret a result or trend.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which explanation is accurate?

- A** The portal color decides treatment.
- B** The flag needs clinical context.
- C** Every flag confirms serious disease.
- D** Every flag is harmless.

2. Which statement preserves action status?

- A** A repeat is being considered but has not been ordered.
- B** A repeat was completed.
- C** The repeat was normal.
- D** A repeat has ruled out disease.

3. Which reassurance is unsupported?

- A** The flag is not itself a diagnosis.
- B** Previous results may provide context.
- C** There is definitely nothing to worry about.
- D** We will review the symptoms.

4. What is a working diagnosis?

- A** An administrative flag
- B** Proof that no further review is needed
- C** A provisional explanation
- D** Every abnormal test

Answers and explanations: [page 91](#).

Outside the range is not the diagnosis

E Conversations / turns 1-10

04

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

preanalytical variation / pending result / confirmatory test / trend / working diagnosis / reference interval / clinical significance / out-of-range flag / baseline / repeat sample

- 01 **Casey:** The portal highlighted my blood result. I saw the warning before anyone explained what it meant.
- 02 **Dr Vega:** I understand why the ¹ _____ worried you. Let us separate the marker from its clinical meaning.
- 03 **Casey:** Does outside the range mean that I definitely have the condition I read about online?
- 04 **Dr Vega:** Not by itself. The ² _____ helps comparison, but we must interpret the result in context.
- 05 **Casey:** What context do you need before you can tell me more about this particular result?
- 06 **Dr Vega:** Your symptoms, earlier results, and the sample circumstances help us assess its ³ _____.
- 07 **Casey:** I remember having the same test previously. Will you compare those reports rather than only this one?
- 08 **Dr Vega:** Yes, where they are comparable. A ⁴ _____ can add information that one isolated result cannot provide.
- 09 **Casey:** I do not know whether the earlier value was typical for me or just another unusual day.
- 10 **Dr Vega:** We should not assume it is your ⁵ _____ without reviewing the relevant history and circumstances.

The conversation, continued

E Conversations / turns 11-20

04

Continue with the word bank on page 37.

- 11 **Casey:** Could something about the sample affect the result, or would that just be an excuse to ignore it?
- 12 **Dr Vega:** Sample context can matter. Reviewing ⁶ _____ does not mean declaring that the laboratory made an error.
- 13 **Casey:** Are you ordering the test again now, or is that only something we might discuss?
- 14 **Dr Vega:** A ⁷ _____ is under consideration. I have not ordered it in this discussion.
- 15 **Casey:** So there is no new result waiting somewhere that I have failed to look at?
- 16 **Dr Vega:** Correct. A ⁸ _____ would be a different situation from considering whether to repeat the test.
- 17 **Casey:** I would rather hear that something is uncertain than be reassured today and surprised tomorrow.
- 18 **Dr Vega:** That is reasonable. A ⁹ _____ must be described as provisional while we evaluate the evidence.
- 19 **Casey:** I understand the difference now: the flag needs review, but it is not the diagnosis itself.
- 20 **Dr Vega:** Exactly. We will explain the plan, including whether a ¹⁰ _____ is appropriate and who will communicate any subsequent result.

ANSWERS / EXPLANATIONS: PAGE 92

1. out-of-range flag | 2. reference interval | 3. clinical significance | 4. trend | 5. baseline | 6. preanalytical variation | 7. repeat sample | 8. pending result | 9. working diagnosis | 10. confirmatory test

A result still awaiting review

F Say it

Sam sees a report in the portal. Dr Ames has received it but has not completed the clinical review. The report is available; its clinical meaning and next action remain to be discussed.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

action / diagnosis / available / completed

Sam: "The report is now ¹ _____."

Dr Ames: "The clinical review is not ² _____."

Sam: "A visible result is not a confirmed ³ _____."

Dr Ames: "We still need to discuss the next ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 92

1. available | 2. completed | 3. diagnosis | 4. action

Rehearse with a fixed script

1. Read the corrected results discussion. Contrast flag, interpretation, and diagnosis.
2. Swap roles. Repeat the lines distinguishing a possible repeat, an order, and a pending result.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A recommendation without a dismissal

A Read the situation

Explain an assessed recommendation while acknowledging the expectation behind a treatment request.

05

After an assessment, Dr Rao has not identified an indication for an antibiotic in this fictional visit. Jules expected one because a previous illness improved after antibiotics. The case does not provide diagnostic findings, a named infection, or medicine instructions. The conversation explains the current recommendation, distinguishes sequence from proof of benefit, and includes an individualized follow-up and safety-net discussion with the actual clinician.

PEOPLE / Dr Rao: General practitioner | Jules: Patient

ACKNOWLEDGE THE EXPECTATION BEFORE EXPLAINING DISAGREEMENT

The patient may be asking for relief, reassurance, or a way to return to work. Address that need without promising an unnecessary treatment or making the conversation a contest.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What is the supplied recommendation based on?

- A A universal ban on antibiotics
- B Jules's employer deciding treatment
- C A laboratory value invented by the learner
- D Dr Rao's assessment in this visit

2. Why did Jules expect an antibiotic?

- A A completed hospital admission today
- B A previous illness improved after antibiotics
- C A new prescription already issued
- D A confirmed allergy

3. What is not supplied?

- A A patient expectation
- B A clinician recommendation
- C A previous experience
- D A named diagnosis or medicine instruction

Answers and explanations: [page 93](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

antibiotic stewardship

Responsible use of antibiotics to preserve benefit and limit avoidable harm.

IN USE support antibiotic stewardship

antibiotic

Medicine used against susceptible bacterial infections.

IN USE discuss an antibiotic recommendation

viral illness

Illness caused by a virus.

IN USE explain a viral illness

bacterial infection

Infection caused by bacteria.

IN USE assess a suspected bacterial infection

self-limiting illness

Illness that can resolve without a treatment directed at its cause.

IN USE explain a self-limiting illness

symptom relief

Reduction of an uncomfortable symptom.

IN USE discuss symptom relief

antimicrobial resistance

Ability of microbes to withstand medicines intended to act against them.

IN USE reduce antimicrobial resistance

treatment indication

Clinical reason supporting use of a particular treatment.

IN USE establish a treatment indication

watchful waiting

Planned observation with defined reassessment rather than immediate intervention.

IN USE explain watchful waiting

return precautions

Instructions about changes requiring further medical attention.

IN USE explain return precautions

therapeutic relationship

Working relationship supporting care between clinician and patient.

IN USE maintain the therapeutic relationship

treatment expectation

What a patient anticipates receiving or achieving from treatment.

IN USE explore the treatment expectation

Precision check

Improvement after a medicine does not, by itself, prove improvement because of the medicine. The current recommendation belongs to this assessed encounter.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

expected benefit

Improvement a treatment may produce in a relevant situation.

IN USE explain the expected benefit

potential harm

Possible adverse consequence of an action or treatment.

IN USE discuss potential harm

reassessment

Further evaluation after time or a change in circumstances.

IN USE arrange clinical reassessment

examination finding

Observation established through a clinical examination.

IN USE explain an examination finding

diagnostic threshold

Evidence level used in deciding whether a diagnosis is sufficiently supported.

IN USE clarify a diagnostic threshold

contingency plan

Agreed response if the situation develops differently from expected.

IN USE agree on a contingency plan

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Not prescribing an antibiotic is not the same as offering no care. Actual symptom advice, reassessment arrangements, and warning signs must come from the treating clinician, not an invented exercise protocol.

Phrases you can actually use

C Notice the language / 1 of 2

ACKNOWLEDGE EXPERIENCE

"I can see why your previous experience shaped your expectation."

EXPLAIN THE DISTINCTION

"Improving afterward does not prove the medicine caused the improvement."

INVITE THE CONCERN

"What worries you most about this recommendation?"

EXPLAIN ONGOING CARE

"We still need an appropriate plan for symptoms and follow-up."

STATE THE RECOMMENDATION

"Based on today's assessment, I am not recommending an antibiotic."

KEEP THE NEED VISIBLE

"Your need for relief still matters."

AVOID A CONTEST

"Let us focus on what is likely to help in this situation."

CHECK THE CONTINGENCY

"Let us confirm what changes mean you should seek further help."

After versus because of

After gives sequence; because of asserts a cause. They are not interchangeable evidence claims.

Based on today's assessment

Locates the recommendation in this encounter rather than presenting an absolute rule.

Still matters

Acknowledges the patient's goal even when the requested treatment is not recommended.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which sentence explains the recommendation respectfully?

- A** Your previous experience is imaginary.
- B** I will prescribe one to end the discussion.
- C** I understand the expectation; today's assessment does not support an antibiotic.
- D** Antibiotics never help anyone.

2. Which inference is unjustified?

- A** Dr Rao assessed the patient.
- B** The current recommendation differs from the expectation.
- C** Improvement afterward proves that antibiotics caused it.
- D** Jules expected treatment.

3. What does no antibiotic mean here?

- A** No reassessment can ever be needed
- B** Every future infection is viral
- C** That particular treatment is not recommended
- D** No further care is allowed

4. Which closing is most useful?

- A** Ignore all new symptoms.
- B** Confirm the individualized follow-up and warning-sign plan.
- C** Call only if you can prove a diagnosis.
- D** Nothing can change after today.

Answers and explanations: [page 93](#).

A recommendation without a dismissal

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

05

WORD + PHRASE BANK

treatment indication / expected benefit / symptom relief / contingency plan / Antimicrobial resistance / return precautions / Reassessment / treatment expectation / potential harm / antibiotic stewardship

- 01 **Jules:** Last time I felt like this, antibiotics helped. I expected to leave with the same treatment today.
- 02 **Dr Rao:** I understand that ¹ _____. Based on today's assessment, I am not recommending an antibiotic.
- 03 **Jules:** Are you saying the improvement last time was not real? I remember feeling much better afterward.
- 04 **Dr Rao:** The improvement was real to you. But improvement afterward does not establish the medicine's ² _____ in this situation.
- 05 **Jules:** I mainly need to feel better and get back to work. I do not want to be dismissed.
- 06 **Dr Rao:** Your need for ³ _____ matters. Not recommending this medicine is not the same as offering no care.
- 07 **Jules:** Then please explain what separates my request from a clinical reason to prescribe the medicine.
- 08 **Dr Rao:** A ⁴ _____ depends on the current assessment and evidence, not only on an earlier prescription.
- 09 **Jules:** Is the concern just about avoiding unnecessary cost, or can an unnecessary medicine cause problems too?
- 10 **Dr Rao:** There can be ⁵ _____, as well as a lack of benefit. We should discuss that honestly without overstating either.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 46.

- 11 **Jules:** I have heard people say their bodies become resistant. Is that the right way to describe it?
- 12 **Dr Rao:** ⁶ _____ concerns microbes that withstand medicines. It is not your body becoming resistant to treatment.
- 13 **Jules:** I follow that distinction. I still want to know what happens if the illness does not develop as expected.
- 14 **Dr Rao:** That needs a clear ⁷ _____, including the appropriate route for further help and reassessment.
- 15 **Jules:** Please make that specific to me rather than just saying to come back if I am worried.
- 16 **Dr Rao:** We will discuss individualized ⁸ _____ and check that the instructions and contact route are clear.
- 17 **Jules:** Then a later review would not mean that I had failed to follow today's recommendation?
- 18 **Dr Rao:** Correct. ⁹ _____ responds to the clinical situation; it is not a judgment about you.
- 19 **Jules:** I can accept a different recommendation when I understand the reasoning and know the next step.
- 20 **Dr Rao:** That is the aim of ¹⁰ _____ alongside good care: explain the decision, address your needs, and keep follow-up clear.

ANSWERS / EXPLANATIONS: PAGE 94

1. treatment expectation | 2. expected benefit | 3. symptom relief | 4. treatment indication | 5. potential harm | 6. Antimicrobial resistance | 7. contingency plan | 8. return precautions | 9. Reassessment | 10. antibiotic stewardship

Explain a different treatment expectation

F Say it

Noor requests a scan because a relative received one. Dr Hale has assessed Noor and does not recommend that scan at this visit. The relative's case is not available. Follow-up remains part of care.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

today / follow-up / unavailable / relative

Noor: "My ¹ _____ received a scan."

Dr Hale: "Your assessment does not support that scan ² _____."

Noor: "The other person's clinical details are ³ _____."

Dr Hale: "We still need clear ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 94

1. relative | 2. today | 3. unavailable | 4. follow-up

Rehearse with a fixed script

1. Read the corrected recommendation. Stress after and because of to distinguish sequence from cause.
2. Swap roles. Repeat the acknowledgment, recommendation, and follow-up lines without turning them into a prescribing rule.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

An invitation is not a diagnosis

A Read the situation

Compare screening benefits and limitations while preserving a voluntary, informed choice.

Dana receives a screening invitation and asks Dr Ellis whether it means a disease is suspected. The invitation is not a diagnostic result. Eligibility and the appropriate options need an individual review; no screening schedule or personal risk calculation is supplied. Dana wants to understand possible benefit, false alarms, and subsequent testing before choosing.

PEOPLE / Dr Ellis: General practitioner | Dana: Patient

06

DISCUSS THE TRADEOFF WITHOUT STEERING THROUGH FEAR

Explain what a screening test can and cannot establish. A useful discussion includes the person's priorities and practical implications, not just a slogan that testing is always better.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What prompted the discussion?

A A completed biopsy

B A treatment complication

C A screening invitation

D A confirmed diagnosis

2. What does Dana want explained?

A Benefits, false alarms, and subsequent testing

B A guaranteed outcome

C An invented personal risk score

D A diagnosis without assessment

3. What is not supplied?

A Dana's questions

B The invitation

C A request for explanation

D An individual screening schedule

Answers and explanations: page 95.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

screening

Testing intended to identify possible disease or risk before relevant symptoms prompt diagnosis.

IN USE discuss screening options

diagnostic testing

Investigation intended to establish or explain a suspected clinical condition.

IN USE distinguish diagnostic testing

average risk

Risk category without specified factors that would place someone in a higher category.

IN USE assess average-risk eligibility

shared decision-making

Combining evidence and patient priorities in a care decision.

IN USE support shared decision-making

absolute risk

Chance of an outcome over a stated period in a defined group.

IN USE explain absolute risk

relative risk

Risk in one group compared with risk in another group.

IN USE distinguish relative risk

false alarm

Result suggesting a problem that further investigation does not confirm.

IN USE explain a possible false alarm

overdiagnosis

Detection of a condition that would not have caused harm during a person's lifetime.

IN USE discuss overdiagnosis

eligibility

Whether a person meets the criteria for an option or service.

IN USE review screening eligibility

prevention

Action aimed at reducing disease occurrence or its consequences.

IN USE discuss preventive care

asymptomatic

Without the relevant symptoms being considered.

IN USE describe an asymptomatic population

positive screen

Screening result indicating that further evaluation may be needed.

IN USE explain a positive screen

Precision check

Screening and diagnostic testing serve different purposes. A screening invitation is not evidence that disease has been detected.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

screening interval

Time between planned screening episodes.

IN USE review the screening interval

risk factor

Characteristic associated with a change in likelihood of an outcome.

IN USE identify a relevant risk factor

family history

Health information about a person's biological relatives.

IN USE update the family history

informed choice

Decision made with adequate understanding of relevant options and consequences.

IN USE support an informed choice

decision aid

Resource helping a person compare care options and their tradeoffs.

IN USE use a decision aid

follow-up test

Further investigation after an earlier test or finding.

IN USE explain a follow-up test

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

A risk percentage needs an outcome, time period, and comparison group. Do not invent those details or use a relative change as though it were an absolute chance.

Phrases you can actually use

C Notice the language / 1 of 2

EXPLAIN THE INVITATION

"This invites a discussion; it does not announce a diagnosis."

EXPLAIN AN UNCERTAIN RESULT

"A positive screen may lead to further evaluation."

ASK ABOUT PRIORITIES

"Which parts of the decision matter most to you?"

PROTECT CHOICE

"You can ask questions before deciding."

SEPARATE PURPOSES

"Screening and testing a symptom are different situations."

NAME THE TRADEOFF

"We should discuss possible benefit as well as false alarms."

CLARIFY A NUMBER

"Over what time period does that risk apply?"

REVIEW ELIGIBILITY

"We need to check which options fit your circumstances."

06

May lead to

Describes a possible consequence rather than an inevitable result.

Rather than

Makes the contrast between screening and diagnosis explicit.

Over what period

Requests a missing time horizon needed to interpret a risk statement.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

06

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. What does the invitation establish?

- A** An opportunity to discuss screening
- B** A diagnosis
- C** A positive result
- D** A guarantee of eligibility for every test

2. Which explanation of a positive screen is safest?

- A** It always proves disease.
- B** It always proves a laboratory mistake.
- C** It never changes care.
- D** It may require further evaluation; it is not automatically a diagnosis.

3. Which risk statement is incomplete?

- A** The risk is lower, without an outcome or time period.
- B** The invitation is not a result.
- C** Dana has questions about tradeoffs.
- D** Eligibility needs review.

4. What supports informed choice?

- A** Explain benefits, limitations, and next steps, then elicit priorities.
- B** Hide false alarms to increase participation.
- C** Treat every question as refusal.
- D** Promise that screening prevents every illness.

Answers and explanations: [page 95](#).

An invitation is not a diagnosis

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

screening / eligibility / asymptomatic / Diagnostic testing / shared decision-making / Absolute risk / informed choice / overdiagnosis / false alarm / positive screen

06

- 01 **Dana:** This letter invited me for screening. Does that mean someone thinks I already have the disease?
- 02 **Dr Ellis:** An invitation to ¹ _____ does not announce a diagnosis. Let us review what the invitation offers.
- 03 **Dana:** I do not have the symptom mentioned in an article I read. Is that a contradiction?
- 04 **Dr Ellis:** Not necessarily. Screening often concerns an ² _____ population; investigating a symptom is a different clinical situation.
- 05 **Dana:** Then the test in this letter is not the same as testing to explain a current complaint?
- 06 **Dr Ellis:** Correct. ³ _____ and screening have different purposes, although one can lead to the other.
- 07 **Dana:** Before deciding, I want to know what a worrying result would mean and what would happen afterward.
- 08 **Dr Ellis:** A ⁴ _____ may lead to further evaluation. It does not automatically confirm the condition.
- 09 **Dana:** Could I go through more tests and then discover that the original concern was not confirmed?
- 10 **Dr Ellis:** Yes, a ⁵ _____ is one limitation to explain alongside the possible benefits and other consequences.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 55.

- 11 **Dana:** People also talk about finding conditions that would never have affected them. Is that the same issue?
- 12 **Dr Ellis:** That is ⁶ _____, which differs from a false alarm. We should explain the distinction without assuming either will happen to you.
- 13 **Dana:** I would like the numbers explained carefully. A headline saying lower risk does not tell me much.
- 14 **Dr Ellis:** We need the event and time period. ⁷ _____ is not the same thing as a relative comparison.
- 15 **Dana:** And you still need to check whether this particular screening option applies to my circumstances?
- 16 **Dr Ellis:** Yes. We must review ⁸ _____ and your history rather than treating a general invitation as the entire assessment.
- 17 **Dana:** I want to compare the options without feeling that asking about limitations means I am refusing care.
- 18 **Dr Ellis:** Questions support ⁹ _____. Your priorities and the evidence both belong in the discussion.
- 19 **Dana:** That gives me a clearer starting point. I understand this is an invitation, not a diagnosis or a guarantee.
- 20 **Dr Ellis:** Exactly. We can use the relevant information to support an ¹⁰ _____, then agree on any next step.

06

ANSWERS / EXPLANATIONS: PAGE 96

1. screening | 2. asymptomatic | 3. Diagnostic testing | 4. positive screen | 5. false alarm | 6. overdiagnosis | 7. Absolute risk | 8. eligibility | 9. shared decision-making | 10. informed choice

Clarify a population statistic

F Say it

A leaflet gives a risk comparison for a defined study population over five years. Kim asks whether it is a personal prediction. Dr Reed explains that individual interpretation requires clinical context.

Complete the four gaps. Use each bank entry once.

06

WORD + PHRASE BANK

population / prediction / context / years

Kim: "The leaflet describes a study ¹ _____."

Dr Reed: "Its time horizon is five ² _____."

Kim: "It is not a guaranteed personal ³ _____."

Dr Reed: "Individual interpretation needs clinical ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 96

1. population | 2. years | 3. prediction | 4. context

Rehearse with a fixed script

1. Read the corrected screening exchange. Contrast screening, positive screen, and diagnosis.
2. Swap roles. Repeat the distinction between false alarm and overdiagnosis, preserving the uncertainty.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Make a direct question feel safe

A Read the situation

Ask sensitively about mood and safety while explaining confidentiality limits and a warm handoff.

Chris reports low mood and poor sleep affecting work. Dr Wells will ask about safety directly rather than infer it from appearance. No risk assessment has been completed and no risk level is supplied. A behavioral-health colleague is available for an introduction if appropriate and agreed. Dr Wells must explain relevant confidentiality limits accurately rather than promise absolute secrecy.

PEOPLE / Dr Wells: General practitioner | Chris: Patient

DIRECT CAN ALSO BE COMPASSIONATE

A clear safety question need not be abrupt. Explain why you are asking, listen without judgment, and avoid making disclosure sound like a test the patient can fail.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What impact does Chris report?

- A A completed diagnosis in this case
- C A refusal of all help

- B A known low risk level
- D Mood and sleep difficulties affecting work

2. What remains incomplete?

- A The risk assessment
- C Chris mentioning sleep

- B The appointment booking
- D The colleague's availability

3. What must not be promised?

- A Absolute secrecy
- C An explanation of next steps

- B Respectful listening
- D Attention to the reported difficulties

[Answers and explanations: page 97.](#)

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

low mood

Persistent or recurrent feelings of sadness or reduced emotional well-being.

IN USE explore low mood

anhedonia

Reduced ability to experience interest or pleasure.

IN USE ask about anhedonia

sleep disturbance

Disruption of the usual pattern or quality of sleep.

IN USE describe sleep disturbance

stressor

Circumstance placing psychological or practical demands on a person.

IN USE identify a current stressor

protective factor

Circumstance or resource that may reduce vulnerability to harm.

IN USE explore protective factors

risk assessment

Clinical evaluation of relevant risks and protective circumstances.

IN USE complete a risk assessment

suicidal thoughts

Thoughts about ending one's own life.

IN USE ask directly about suicidal thoughts

psychosocial history

Account of psychological and social circumstances relevant to care.

IN USE take a psychosocial history

nonjudgmental language

Wording that explores experience without assigning blame or moral fault.

IN USE use nonjudgmental language

behavioral health

Care addressing mental health, behavior, and related well-being.

IN USE coordinate behavioral-health care

warm handoff

Supported introduction to another care professional with relevant context.

IN USE arrange a warm handoff

confidentiality limit

Circumstance in which privacy cannot be guaranteed under applicable duties.

IN USE explain confidentiality limits

Precision check

A calm appearance does not establish low risk. Ask and assess directly; do not let an English exercise stand in for an actual safety evaluation.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

crisis response

Immediate professional response to an urgent safety concern.

IN USE activate the appropriate crisis response

distress

Significant emotional suffering or discomfort.

IN USE acknowledge distress

daily functioning

Ability to manage ordinary work, self-care, and other activities.

IN USE assess daily functioning

support network

People or services available to provide help.

IN USE identify a support network

consent to share

Agreement to disclose specified information in an appropriate context.

IN USE clarify consent to share

therapeutic alliance

Collaborative relationship supporting care and agreed goals.

IN USE strengthen the therapeutic alliance

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

A warm handoff is more than passing along a telephone number. Explain the introduction and information sharing, following actual consent and safety requirements.

Phrases you can actually use

C Notice the language / 1 of 2

OPEN SENSITIVELY

"How have things been emotionally as well as physically?"

EXPLAIN THE QUESTION

"I ask directly because your safety matters."

EXPLAIN PRIVACY LIMITS

"I will explain when I may need to involve others for safety."

CLARIFY INFORMATION SHARING

"Let us agree what information is shared, within the applicable duties."

ASK DIRECTLY

"Have you had thoughts about ending your life?"

AVOID AN ASSUMPTION

"I cannot judge that from how you look today."

OFFER AN INTRODUCTION

"We can discuss an introduction to our behavioral-health colleague."

KEEP THE PERSON INVOLVED

"I will explain the next steps as clearly as I can."

Have you had

The present perfect invites experience over a relevant period rather than assuming a current answer.

May need to

Explains a possible duty without promising secrecy or announcing a decision already made.

From how you look

Identifies an inadequate basis for a risk conclusion.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which safety question is direct and neutral?

- A** Have you had thoughts about ending your life?
- B** You would never do anything foolish, would you?
- C** You look fine, so there is no risk.
- D** Can we skip this uncomfortable topic?

2. Which privacy statement is inappropriate?

- A** I promise absolute secrecy in every circumstance.
- B** We should discuss confidentiality limits.
- C** I will explain relevant information sharing.
- D** Your questions about privacy matter.

3. What does a warm handoff add?

- A** A guaranteed cure
- B** A completed risk assessment by definition
- C** An automatic disclosure of everything
- D** A supported introduction with relevant context

4. Which inference must be avoided?

- A** Assessment is still needed.
- B** A calm appearance proves low risk.
- C** Chris reports poor sleep.
- D** Work is affected.

[Answers and explanations: page 97.](#)

Make a direct question feel safe

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

support network / suicidal thoughts / warm handoff / anhedonia / daily functioning / confidentiality limits / nonjudgmental language / consent to share / low mood / risk assessment

07

- 01 **Dr Wells:** You mentioned poor sleep and difficulty at work. How have things been emotionally as well as physically?
- 02 **Chris:** My ¹ _____ has been harder to manage than I expected, but I struggle to explain it.
- 03 **Dr Wells:** We can take it one part at a time. What has changed in your usual activities or interests?
- 04 **Chris:** I stopped enjoying things I normally like. I saw the word ² _____, but I was unsure how to say it.
- 05 **Dr Wells:** Your own description is enough. I also need to ask directly about your safety rather than make assumptions.
- 06 **Chris:** I was worried that my ³ _____ would look normal because I managed to come to this appointment.
- 07 **Dr Wells:** Coming here does not tell me everything. Have you had thoughts about ending your life?
- 08 **Chris:** That is difficult to discuss. Will talking about ⁴ _____ mean that everything I say is automatically shared with everyone?
- 09 **Dr Wells:** No. I will explain privacy and its limits, including when safety duties may require involving other people.
- 10 **Chris:** Please explain those ⁵ _____ before assuming I understand them. I want to answer honestly.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 64.

- 11 **Dr Wells:** Of course. We will discuss the relevant rules and your situation, and I will explain the next steps.
- 12 **Chris:** I appreciate the ⁶ _____. I was afraid this would become a lecture about coping better.
- 13 **Dr Wells:** My aim is to understand what is happening and assess safety, including difficulties and sources of support.
- 14 **Chris:** My sister has been helpful. I would like her considered as part of my ⁷ _____.
- 15 **Dr Wells:** We can discuss that. Our behavioral-health colleague is also available, depending on the assessed needs and agreed plan.
- 16 **Chris:** Could you introduce us? A ⁸ _____ would be easier than repeating everything to a stranger without preparation.
- 17 **Dr Wells:** We can discuss the introduction and what information is appropriate to share, subject to the relevant duties.
- 18 **Chris:** Then my ⁹ _____ is something we clarify rather than assume from my attendance here.
- 19 **Dr Wells:** Yes. We must also complete the safety assessment and respond appropriately to anything urgent that it reveals.
- 20 **Chris:** I understand that the ¹⁰ _____ has not already been completed just because I have started talking.

07

ANSWERS / EXPLANATIONS: PAGE 98

1. low mood | 2. anhedonia | 3. daily functioning | 4. suicidal thoughts | 5. confidentiality limits | 6. nonjudgmental language | 7. support network | 8. warm handoff | 9. consent to share | 10. risk assessment

Introduce support without over-sharing

F Say it

Rae agrees to an introduction to counselor Jo. The relevant information to share will be clarified. Dr Moss has not completed the safety assessment; no risk level is supplied.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

share / established / Jo / complete

Rae: "I agree to meet counselor ¹ _____."

Dr Moss: "We will clarify the information to ² _____."

Rae: "The safety assessment is not ³ _____."

Dr Moss: "No risk level has been ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 98

1. Jo | 2. share | 3. complete | 4. established

Rehearse with a fixed script

1. Read the corrected safety conversation calmly, with a pause before and after the direct question.
2. Swap roles. Repeat the privacy explanation without adding an absolute promise or an invented risk level.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Sent does not mean seen

A Read the situation

Give a referral update that identifies the open question, owner, and next contact.

Dr Byrne referred Fin to a specialist for an unresolved clinical question. The receiving office confirms receipt, but has not confirmed an appointment. Practice coordinator Mina is checking the status and will update Fin on Friday. Friday is a status-update commitment, not a clinical waiting instruction or appointment date. New or worsening concerns must use the practice's actual clinical route.

PEOPLE / Dr Byrne: General practitioner | Fin: Patient

08

MAKE THE STAGE VISIBLE

Patients may understand referred to mean booked or accepted for treatment. Name the actual stage and retain responsibility for unresolved care rather than treating the referral as a transfer of every duty.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What has the specialist office confirmed?

- A** Receipt of the referral
- C** Completed treatment

- B** A Friday appointment
- D** A resolved diagnosis

2. Who is checking the status?

- A** Nobody
- C** Fin's employer

- B** Mina
- D** A clinician not named in the case

3. What does Friday mean?

- A** A guaranteed appointment
- C** A completed consultation

- B** Permission to wait with worsening symptoms
- D** A promised status update

Answers and explanations: [page 99](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

referral

Request for another professional or service to assess or assist.

IN USE send a specialist referral

consultation question

Specific issue the referring clinician asks the receiving service to address.

IN USE state the consultation question

urgency category

Assigned level of time sensitivity under a clinical process.

IN USE verify the urgency category

referral receipt

Confirmation that the receiving service has obtained the referral.

IN USE confirm referral receipt

appointment confirmation

Verification that a specific appointment has been arranged.

IN USE obtain appointment confirmation

closed-loop referral

Referral process tracking receipt, follow-up, and information returned to the referrer.

IN USE complete a closed-loop referral

consultant letter

Communication from a specialist summarizing an assessment or advice.

IN USE review the consultant letter

awaiting review

Received but not yet evaluated by the relevant reviewer.

IN USE mark a referral as awaiting review

missed appointment

Scheduled encounter that did not take place as arranged.

IN USE follow up a missed appointment

results ownership

Responsibility for reviewing and acting on investigation results.

IN USE clarify results ownership

pending action

Task still requiring completion.

IN USE identify a pending action

acknowledgment

Confirmation that a message or request was received.

IN USE obtain an acknowledgment

08

Precision check

Receipt, review, booking, attendance, and a returned clinical report are different stages. Do not report a later stage when only an earlier one is confirmed.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

interim care

Care provided while another assessment or transition is pending.

IN USE clarify interim care

contact preference

Person's preferred appropriate method of communication.

IN USE verify the contact preference

escalation route

Process for raising an unresolved or urgent concern to the right professional.

IN USE use the clinical escalation route

care transition

Movement of care between settings, professionals, or stages.

IN USE support a care transition

record transfer

Appropriate movement of relevant information between care providers.

IN USE verify record transfer

access barrier

Obstacle that makes obtaining a service difficult.

IN USE identify an access barrier

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

A status-update date must never imply that symptoms can safely wait until that date. Keep administrative progress and clinical escalation separate.

Phrases you can actually use

C Notice the language / 1 of 2

STATE THE CONFIRMED STAGE

"The receiving office has confirmed receipt."

IDENTIFY THE OWNER

"Mina is checking the status."

KEEP CARE CONNECTED

"We need to clarify who handles the outstanding clinical questions."

CONFIRM CONTACT

"Which approved contact method should we use for the update?"

NAME THE MISSING STEP

"An appointment has not yet been confirmed."

DEFINE THE DATE

"Friday is our update date, not your appointment date."

SEPARATE URGENT CONCERNS

"New or worsening symptoms use the clinical route, not a routine status message."

CLOSE THE LOOP

"We will track the response and the information returned after consultation."

08

Has confirmed versus will check

Separates a completed acknowledgment from a future task.

Not yet

Marks an unfinished stage without implying that it has been refused.

Update date versus appointment date

Prevents an administrative commitment from becoming a false booking promise.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which update is supported?

- A** Receipt is confirmed; an appointment is not.
- B** Fin will be seen Friday.
- C** Treatment is complete.
- D** The clinical question is resolved.

2. Which phrase could dangerously confuse two processes?

- A** Friday is a status update.
- B** Mina is checking the referral.
- C** Clinical concerns use the clinical route.
- D** Wait until Friday with any new symptoms.

3. What makes the next step accountable?

- A** A vague promise that someone will look
- B** An assumed appointment
- C** A closed record before review
- D** A named owner and a defined update

4. What remains important after sending a referral?

- A** Ignoring the consultation question
- B** Following the response and clarifying continuing responsibilities
- C** Assuming all duties disappeared
- D** Declaring attendance complete

Answers and explanations: [page 99](#).

Sent does not mean seen

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

interim care / consultation question / closed-loop referral / escalation route / pending action / record transfer / contact preference / referral receipt / Appointment confirmation / consultant letter

08

- 01 **Fin:** I was told the referral had gone through. Does that mean the specialist has booked me in?
- 02 **Dr Byrne:** We have ¹ _____, but an appointment has not yet been confirmed.
- 03 **Fin:** Thank you for separating those. I heard gone through and assumed there was a date somewhere.
- 04 **Dr Byrne:** ² _____ is a separate step. Mina is checking the status with the receiving office.
- 05 **Fin:** What exactly is the specialist being asked to address? I do not want to arrive without understanding that.
- 06 **Dr Byrne:** We will review the ³ _____ with you and make sure the relevant records accompany the request.
- 07 **Fin:** I can explain my concern, but I may not remember every detail that is already in your notes.
- 08 **Dr Byrne:** Appropriate ⁴ _____ helps the receiving team. It does not replace listening to your current account.
- 09 **Fin:** Mina mentioned Friday. Is that when the consultation will happen, or when someone will contact me?
- 10 **Dr Byrne:** Friday is a status update. The ⁵ _____ is checking progress, not attending a confirmed appointment.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 73.

- 11 **Fin:** Who should deal with questions before the specialist sees me? I do not want to fall between teams.
- 12 **Dr Byrne:** We must clarify ⁶ _____ and the outstanding responsibilities rather than assume everything transferred when we sent the request.
- 13 **Fin:** If something changes clinically before Friday, should I wait for Mina's administrative update to mention it?
- 14 **Dr Byrne:** No. Use the actual clinical ⁷ _____ for new or worsening concerns. The update date is not advice to wait.
- 15 **Fin:** I also need calls at a time I can answer privately. Can we agree on how the update reaches me?
- 16 **Dr Byrne:** Yes, we will verify your ⁸ _____ and use the practice's approved communication process.
- 17 **Fin:** Once the appointment happens, will the specialist's advice come back here, or do I have to relay everything?
- 18 **Dr Byrne:** We will track the ⁹ _____ and review the responsibilities it identifies, rather than assume attendance alone closes every question.
- 19 **Fin:** That is clearer: receipt now, a status update Friday, and no appointment date promised yet.
- 20 **Dr Byrne:** Exactly. A ¹⁰ _____ keeps the stages and responsibilities connected until the relevant follow-up is complete.

08

ANSWERS / EXPLANATIONS: PAGE 100

1. referral receipt | 2. Appointment confirmation | 3. consultation question | 4. record transfer | 5. pending action | 6. interim care | 7. escalation route | 8. contact preference | 9. consultant letter | 10. closed-loop referral

Track a returned specialist report

F Say it

A specialist consultation occurred Monday. The report has not arrived. Coordinator Zoe is requesting it and will update the referring team Wednesday. No new treatment instruction is supplied.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

supplied / Monday / arrived / Wednesday

Clinician: "The consultation occurred on ¹ _____."

Zoe: "The report has not ² _____."

Clinician: "The update is due ³ _____."

Zoe: "No new treatment instruction has been ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 100

1. Monday | 2. arrived | 3. Wednesday | 4. supplied

Rehearse with a fixed script

1. Read the corrected referral update. Stress receipt, appointment, and status update as separate stages.
2. Swap roles. Repeat the distinction between Friday's administrative update and the clinical escalation route.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A consultation through a qualified interpreter

E More workplace conversations / 1-10

Speak directly to the patient, use manageable segments, and verify the interpreted message.

Dr Patel meets Noor with qualified interpreter Lin. The script shows the clinician and interpreter coordinating the interpreted consultation; Noor remains the patient and decision-maker. A relative has offered to summarize, but the service provides qualified interpretation. No clinical diagnosis or treatment choice is supplied.

PEOPLE / Dr Patel: General practitioner | Lin: Qualified interpreter

Use each bank entry once, without changing its form. Answers follow turn 20.

09

WORD + PHRASE BANK

closing summary / source attribution / teach-back / clarification / direct address / qualified interpretation

- 01 Dr Patel:** Before we begin, I will introduce your role to Noor and confirm the language and communication preferences.
- 02 Lin:** Thank you. Please speak directly to Noor rather than asking me what the patient thinks in the third person.
- 03 Dr Patel:** I will use ¹ _____ and keep my attention on the patient, even while you interpret the words.
- 04 Lin:** That helps preserve the relationship. Please also give me manageable segments rather than a long uninterrupted explanation.
- 05 Dr Patel:** I can pause after one idea. I do not want a long speech to become an abbreviated summary.
- 06 Lin:** I will interpret the content rather than replace it with my own advice or a shortened conclusion.
- 07 Dr Patel:** If a term is ambiguous, please identify the need for ² _____ before we continue to a new point.
- 08 Lin:** I will make that transparent to both of you rather than silently choose a meaning that may be wrong.
- 09 Dr Patel:** Noor's relative offered to summarize. How should I explain why we are using the service today?
- 10 Lin:** Explain the value of accurate, confidential language support without suggesting that the relative has done something wrong.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 76.

- 11 **Dr Patel:** I will describe ³ _____ as support for Noor's participation, not as a substitute decision-maker.
- 12 **Lin:** Exactly. I do not provide consent on the patient's behalf or decide which clinical option the patient should choose.
- 13 **Dr Patel:** When I check understanding, I will ask Noor to explain the message in their own words.
- 14 **Lin:** I can interpret that response fully. Please do not treat a brief yes as proof that the explanation was clear.
- 15 **Dr Patel:** That makes ⁴ _____ a check on my explanation, not a test for you to answer instead of Noor.
- 16 **Lin:** Yes. If the response shows a misunderstanding, explain the point differently and let me interpret the revised explanation.
- 17 **Dr Patel:** I will also record that language support was used and distinguish the patient account from anyone else's report.
- 18 **Lin:** That preserves ⁵ _____ while keeping the interpreter's role clear.
- 19 **Dr Patel:** Before closing, I will confirm the actual next step and the appropriate language support for follow-up.
- 20 **Lin:** A clear ⁶ _____ gives Noor a chance to correct the meaning before the consultation ends.

09

ANSWERS / EXPLANATIONS: PAGE 101

1. direct address | 2. clarification | 3. qualified interpretation | 4. teach-back | 5. source attribution | 6. closing summary

The interpreter requests a shorter segment

F Say it in a new situation

The clinician gives a long explanation. The interpreter asks for one idea at a time so the full meaning can be interpreted. No summary is substituted.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

pause / decision-maker / idea / meaning

09

Interpreter: "Please give one ¹ _____ at a time."

Clinician: "I will ² _____ between them."

Interpreter: "I will preserve the full ³ _____."

Clinician: "The patient remains the ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 101

1. idea | 2. pause | 3. meaning | 4. decision-maker

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

A work note without unnecessary disclosure

E More workplace conversations / 1-10

Clarify the purpose and authorized content of a work-related letter.

Patient Ellis asks Dr Shaw for a work note. The employer requests a diagnosis, but Ellis wants to discuss what information is actually necessary and authorized. The clinician will follow applicable rules and use supported facts. No work-capacity judgment or legal entitlement is supplied.

PEOPLE / Ellis: Patient | Dr Shaw: General practitioner

Use each bank entry once, without changing its form. Answers follow turn 20.

10

WORD + PHRASE BANK

evidence limitation / functional information / authorized recipient / job demands / disclosure scope / occupational recommendation

- 01 **Ellis:** My employer wants a note and asked for my diagnosis. I am not comfortable sending my whole medical history.
- 02 **Dr Shaw:** Let us clarify the purpose of the letter and what information is necessary under the applicable process.
- 03 **Ellis:** I want them to understand the relevant limitation, but not receive details unrelated to work.
- 04 **Dr Shaw:** We should distinguish ¹ _____ from a full clinical history, while checking what this particular process actually requires.
- 05 **Ellis:** Could the letter say I cannot do a particular task even if we have not assessed that task?
- 06 **Dr Shaw:** No. Any statement needs a basis in the actual assessment; I should not invent a restriction to make the letter sound useful.
- 07 **Ellis:** Then we need to discuss the real duties rather than use a generic phrase about all work.
- 08 **Dr Shaw:** Yes. The ² _____ provide context, but the clinical assessment and any authorized occupational review remain separate.
- 09 **Ellis:** If the employer asks for more information later, does my request for this note authorize every future disclosure?
- 10 **Dr Shaw:** It does not automatically do that. We need the appropriate permission and applicable rules for the information being shared.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 79.

- 11 **Ellis:** I would like to know the recipient and see what the note is intended to communicate.
- 12 **Dr Shaw:** We will clarify the ³ _____ rather than treat a request for a note as unlimited permission.
- 13 **Ellis:** Can the note guarantee that the employer will approve my preferred arrangement?
- 14 **Dr Shaw:** No. The letter can communicate supported information; the employer's process and any legal rights need their own appropriate handling.
- 15 **Ellis:** I understand. I do not want an unsupported promise about the employment decision.
- 16 **Dr Shaw:** We should also distinguish an ⁴ _____ from the employer's eventual decision.
- 17 **Ellis:** What if the form has a box that does not match what you can establish?
- 18 **Dr Shaw:** We should explain the ⁵ _____ or seek clarification rather than force an unsupported answer.
- 19 **Ellis:** Then the note can stay factual, limited to its purpose, and shared through the agreed route.
- 20 **Dr Shaw:** Exactly. A clear ⁶ _____ and supported content matter more than adding unnecessary medical detail.

10

ANSWERS / EXPLANATIONS: PAGE 102

1. functional information | 2. job demands | 3. disclosure scope | 4. occupational recommendation | 5. evidence limitation | 6. authorized recipient

The employer asks for an unrelated history

F Say it in a new situation

A work-note request asks for unrelated past diagnoses. The patient has not authorized that disclosure. The clinician checks the purpose and applicable requirements before responding.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

authorized / requirements / permission / unrelated

10

Patient: "Those past diagnoses are ¹ _____ to the stated purpose."

Clinician: "Disclosure has not been ² _____."

Patient: "Please clarify the actual ³ _____."

Clinician: "We will not assume unlimited ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 102

1. unrelated | 2. authorized | 3. requirements | 4. permission

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

Closing an unanswered result call

E More workplace conversations / 1-10

Assign follow-up responsibility without treating an attempted call as completed communication.

Dr Wynn and coordinator Jo review a result that still needs the clinician's explanation to the patient. A call went unanswered and no sensitive voicemail was left because the contact permissions require checking. The actual clinical priority and escalation route must be confirmed; no result value or safe delay is supplied.

PEOPLE / Dr Wynn: General practitioner | Jo: Care coordinator

Use each bank entry once, without changing its form. Answers follow turn 20.

WORD + PHRASE BANK

unsuccessful contact attempt / clinical priority / acknowledged escalation / follow-up ownership / contact permission / closed-loop follow-up

11

- 01 **Jo:** The call went unanswered, and the result has not yet been explained to the patient. I have recorded the attempt.
- 02 **Dr Wynn:** Thank you. An attempted call is not completed communication. We need the actual priority and follow-up process.
- 03 **Jo:** I did not leave sensitive details because the voicemail permission was not clear in the record.
- 04 **Dr Wynn:** We should verify the ¹ _____ rather than assume that a listed number permits every kind of message.
- 05 **Jo:** The task currently says called patient, which might make another colleague think the discussion already happened.
- 06 **Dr Wynn:** Change the status through the approved record process to reflect an ² _____, without hiding the original history.
- 07 **Jo:** Who should determine how urgently the next attempt or escalation needs to happen?
- 08 **Dr Wynn:** The responsible clinician must confirm the clinical priority using the actual result and circumstances, not an administrative guess.
- 09 **Jo:** I will not infer that it can wait merely because the patient did not answer.
- 10 **Dr Wynn:** Correct. The ³ _____ remains a clinical question requiring the appropriate review.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 82.

- 11 **Jo:** Once that is confirmed, we need a named person to carry out the next agreed contact step.
- 12 **Dr Wynn:** Let us assign ⁴ _____ explicitly and record the actual plan and route.
- 13 **Jo:** If that person is away, should the task remain in an individual inbox without review?
- 14 **Dr Wynn:** No. Use the service's actual coverage and escalation arrangements so the responsibility does not depend on an unmonitored inbox.
- 15 **Jo:** We also need to know when the receiving clinician has accepted any escalation.
- 16 **Dr Wynn:** An ⁵ _____ is different from a sent message with no response.
- 17 **Jo:** I will keep the result explanation marked incomplete until the appropriate discussion or documented process is completed.
- 18 **Dr Wynn:** Yes. Record what was actually communicated, by whom, and any unresolved issue according to the clinical process.
- 19 **Jo:** That should prevent another colleague from assuming that the patient knows the result just because we tried calling.
- 20 **Dr Wynn:** Exactly. A ⁶ _____ depends on accurate status, assigned responsibility, and the actual clinical response.

11

ANSWERS / EXPLANATIONS: PAGE 103

1. contact permission | 2. unsuccessful contact attempt | 3. clinical priority | 4. follow-up ownership | 5. acknowledged escalation | 6. closed-loop follow-up

A message was sent but not accepted

F Say it in a new situation

A coordinator sends an escalation to the appropriate clinician. No acknowledgment has arrived. The task remains open under the actual service process.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

process / open / sent / acknowledged

Coordinator: "The escalation was ¹ _____."

Clinician: "Receipt has not been ² _____."

Coordinator: "The task remains ³ _____."

Clinician: "We must follow the actual service ⁴ _____."

11

ANSWERS / EXPLANATIONS: PAGE 103

1. sent | 2. acknowledged | 3. open | 4. process

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

01 Briefing and language checks

Agreeing on a consultation agenda

A Briefing checks / page 4

1. What was originally planned?

C. A long-term-condition review

The booking is a review; the other concerns emerge during this consultation.

2. Which concern does Morgan prioritize?

B. Tiredness

Morgan explicitly identifies tiredness as the concern that matters most.

3. What has not yet happened?

C. Clinical assessment of urgency

The brief says urgency has not been assessed at this point.

D Language checks / page 9

1. Which response avoids dismissing the symptom?

D. Let us clarify the tiredness before deciding the agenda.

Clarification must precede assumptions about priority or the symptom's significance.

2. Which wording preserves the patient's preference?

D. Morgan identifies tiredness as the main concern.

The statement records Morgan's priority without inventing a refusal or diagnosis.

3. What does follow-up mean here?

D. An agreed later review of unfinished work

A follow-up needs a clear purpose and does not replace necessary assessment.

4. Which signpost is clearest?

B. First the tiredness history, then we will agree on the remaining plan.

Naming the history before the planning step makes the conversational sequence clear.

01 Conversation answers

Agreeing on a consultation agenda

E Dialogue gaps

Pages 10-11

- 1. presenting concern.** Turn 2. The presenting concern is the problem Morgan brings for assessment today.
- 2. consultation agenda.** Turn 4. The consultation agenda includes the issues discussed and prioritized for this encounter.
- 3. priority.** Turn 6. Priority identifies what needs attention first rather than what happens to be written on a booking.
- 4. functional limitation.** Turn 8. Functional limitation describes difficulty with usual activities rather than a diagnosis by itself.
- 5. long-term condition.** Turn 10. A long-term condition is an ongoing health issue requiring continuing care and review.
- 6. signposting.** Turn 12. Signposting tells the listener which part of the conversation comes next and why.
- 7. follow-up visit.** Turn 14. A follow-up visit provides an agreed later encounter for review or unfinished care.
- 8. safety-netting.** Turn 16. Safety-netting explains changes that require further help and the route for obtaining it.
- 9. shared agenda.** Turn 18. A shared agenda reflects both the patient's concerns and the clinician's assessed priorities.
- 10. continuity of care.** Turn 20. Continuity of care connects this visit with ongoing review instead of treating every concern as unrelated.

F Transfer conversation

Page 12

1. review

Dr Chen: "The planned visit is a blood-pressure review."

The planned appointment is the review named in the scenario.

2. sleep

Lee: "I also want to discuss my sleep."

Sleep is the new concern Lee raises during this encounter.

3. clarify

Dr Chen: "I will first clarify that concern."

Clarification comes before deciding the clinical priorities in this scenario.

4. unfinished

Lee: "The travel form remains unfinished."

The form has not been completed or removed from the plan.

02 Briefing and language checks

Taking a precise symptom history

A Briefing checks / page 13

1. When did the episodes begin according to Ari?

D. Tuesday

Tuesday is the onset Ari reports, not a date independently established by testing.

2. What sensation does Ari describe?

B. Feeling about to faint

The brief distinguishes Ari's near-faint feeling from room-spinning language.

3. What remains unclear?

B. Episode duration

Duration is specifically unresolved and should not be silently filled in.

D Language checks / page 18

1. Which sentence distinguishes onset from duration?

C. Episodes began Tuesday; the length of each is still unclear.

Tuesday gives the onset; stating that episode length is unclear preserves the unresolved duration.

2. Which phrase overstates the evidence?

D. Standing definitely caused the condition.

An association in the history does not by itself demonstrate causation.

3. Which question is least leading?

B. What happens during an episode?

The open question invites Ari's account without embedding an unsupported conclusion.

4. What is a pertinent negative?

B. A relevant absence established by asking or assessing

An unmentioned symptom cannot be documented as denied without an appropriate question or assessment.

02 Conversation answers

Taking a precise symptom history

E Dialogue gaps

Pages 19-20

- 1. onset.** Turn 2. Onset identifies when Ari first noticed the episodes rather than how long each episode lasts.
- 2. episodic.** Turn 4. Episodic means the sensation occurs in separate periods rather than continuously all the time.
- 3. trigger.** Turn 6. A trigger is a circumstance associated with starting the symptom, not proof of its underlying cause.
- 4. relieving factor.** Turn 8. A relieving factor is something the patient reports as reducing the sensation during an episode.
- 5. duration.** Turn 10. Duration is the length of a single episode, which remains unclear in this history.
- 6. associated symptom.** Turn 12. An associated symptom occurs alongside the main concern and needs its own accurate history.
- 7. frequency.** Turn 14. Frequency tells us how often episodes occur, separate from onset, severity, or the length of one episode.
- 8. chronology.** Turn 16. Chronology preserves the order of reported events without rearranging them to fit a presumed explanation.
- 9. diagnostic uncertainty.** Turn 18. Diagnostic uncertainty means the cause is not established, even though the symptom account is becoming clearer.
- 10. pertinent negative.** Turn 20. A pertinent negative is a relevant absence established through questioning or assessment, not through silence.

F Transfer conversation

Page 21

1. Friday

Dr Park: "The reported onset was Friday."

Friday identifies when Jamie says the intermittent tingling first began.

2. two

Jamie: "One episode lasts about two minutes."

Two is Jamie's estimate of a single episode's duration.

3. intermittent

Dr Park: "The symptoms are intermittent."

Intermittent describes episodes that occur with intervals between them.

4. established

Dr Park: "The cause is not yet established."

The scenario supplies a symptom account but no confirmed explanation.

03 Briefing and language checks

Reconciling medicines without blame

03

A Briefing checks / page 22

1. Which records differ?

A. The clinic and hospital lists

The case explicitly identifies a discrepancy between the two medicine lists.

2. What barrier does Robin report?

C. Cost

Robin reports missed medicine because of cost, not a verified clinical instruction.

3. What has not been verified?

C. Whether the products share an ingredient

The case does not establish the contents or overlap of the products.

D Language checks / page 27

1. Which question avoids blame?

D. Has cost made the plan difficult to follow?

Asking about cost explores a practical barrier without assuming motives or misconduct.

2. Which conclusion is unsupported?

D. Different brands prove different ingredients.

Brand names alone do not establish the active ingredients or possible overlap.

3. Which distinction matters?

B. Actual use versus recorded instructions

Recorded instructions do not establish what a patient actually takes at home.

4. What should happen before finalizing the list?

A. Verification of conflicting information

Verification is needed because neither conflicting record has been established as current.

03 Conversation answers

Reconciling medicines without blame

E Dialogue gaps

Pages 28-29

- 1. over-the-counter product.** Turn 2. An over-the-counter product is bought without a prescription and still belongs in a medicine review.
- 2. medicine list.** Turn 4. A medicine list records products and instructions but does not prove current actual use.
- 3. medication reconciliation.** Turn 6. Medication reconciliation compares records with actual use and resolves discrepancies rather than merely copying a list.
- 4. brand name.** Turn 8. A brand name is a commercial product name and does not by itself identify a different medicinal ingredient.
- 5. active ingredient.** Turn 10. The active ingredient is the medicinal substance being compared to investigate possible product overlap.
- 6. adherence.** Turn 12. Adherence describes actual use in relation to the agreed plan and can be affected by practical barriers.
- 7. intolerance.** Turn 14. Intolerance means difficulty tolerating a medicine and is not automatically an immune-mediated allergy.
- 8. drug allergy.** Turn 16. A drug allergy refers to an immune-mediated reaction and needs accurate history rather than an assumed label.
- 9. record discrepancy.** Turn 18. A record discrepancy is an unresolved difference between medicine records that needs verification.
- 10. medication review.** Turn 20. The medication review evaluates actual use, product information, barriers, and unresolved clinical questions together.

F Transfer conversation

Page 30

1. prescriber

Taylor: "An outside prescriber gave the instruction."

The reported instruction comes from the outside prescriber in this case.

2. includes

Dr Noor: "The local list still includes the medicine."

The unresolved local entry is explicitly described as still including the medicine.

3. unknown

Taylor: "The instruction date is unknown."

The date has not been supplied and must not be guessed.

4. verify

Dr Noor: "I will verify the instruction."

Verification is required before treating the reported instruction as settled.

04 Briefing and language checks

Explaining an uncertain test result

04

A Briefing checks / page 31

1. What has Casey seen?

C. An out-of-range flag

The brief supplies a laboratory flag but no confirmed diagnosis or numerical value.

2. What is the repeat-test status?

D. Under consideration

A repeat is being considered and has not yet been ordered.

3. What will Dr Vega use alongside the result?

D. Symptoms, previous results, and sample context

Interpretation requires the stated clinical and sample context, not the flag alone.

D Language checks / page 36

1. Which explanation is accurate?

B. The flag needs clinical context.

A laboratory comparison marker cannot replace interpretation in the person's clinical context.

2. Which statement preserves action status?

A. A repeat is being considered but has not been ordered.

The proposed test has not advanced to an order or completed result.

3. Which reassurance is unsupported?

C. There is definitely nothing to worry about.

The case supplies no basis for guaranteeing that the result has no clinical importance.

4. What is a working diagnosis?

C. A provisional explanation

A working diagnosis remains provisional and may change as evidence develops.

04 Conversation answers

Explaining an uncertain test result

E Dialogue gaps

Pages 37-38

- 1. out-of-range flag.** Turn 2. An out-of-range flag compares a result with a stated interval without establishing a diagnosis.
- 2. reference interval.** Turn 4. A reference interval is a laboratory comparison range, not a stand-alone diagnosis for an individual patient.
- 3. clinical significance.** Turn 6. Clinical significance concerns what the finding means for care, beyond whether the laboratory flagged it.
- 4. trend.** Turn 8. A trend is the pattern across comparable results over time rather than one isolated flagged value.
- 5. baseline.** Turn 10. A baseline is an earlier or usual state used for comparison and must be established appropriately.
- 6. preanalytical variation.** Turn 12. Preanalytical variation concerns differences arising before analysis, which may be relevant without proving an error occurred.
- 7. repeat sample.** Turn 14. A repeat sample would be a new specimen for another test; here it remains a possibility rather than an order.
- 8. pending result.** Turn 16. A pending result belongs to an investigation whose outcome is not yet available, not to a test merely being considered.
- 9. working diagnosis.** Turn 18. A working diagnosis is a provisional explanation and should be distinguished from a confirmed conclusion.
- 10. confirmatory test.** Turn 20. A confirmatory test helps establish a suspected finding when clinically appropriate; none is automatically required by a flag alone.

F Transfer conversation

Page 39

1. available

Sam: "The report is now available."

The portal report exists and can be seen in this scenario.

2. completed

Dr Ames: "The clinical review is not completed."

Receiving a report does not mean its clinical review has been completed.

3. diagnosis

Sam: "A visible result is not a confirmed diagnosis."

The scenario provides a report but does not establish a diagnosis.

4. action

Dr Ames: "We still need to discuss the next action."

The next clinical action remains unresolved in the supplied facts.

05 Briefing and language checks

Responding to an antibiotic request

A Briefing checks / page 40

05

1. What is the supplied recommendation based on?

D. Dr Rao's assessment in this visit

The case states an assessed recommendation without supplying a general prescribing rule.

2. Why did Jules expect an antibiotic?

B. A previous illness improved after antibiotics

Jules connects improvement in an earlier illness with having taken antibiotics.

3. What is not supplied?

D. A named diagnosis or medicine instruction

The dialogue must not invent diagnostic findings or medicine instructions absent from the case.

D Language checks / page 45

1. Which sentence explains the recommendation respectfully?

C. I understand the expectation; today's assessment does not support an antibiotic.

Acknowledging the expectation respects experience while retaining the recommendation based on today's assessment.

2. Which inference is unjustified?

C. Improvement afterward proves that antibiotics caused it.

Timing alone cannot establish that a medicine caused an earlier recovery.

3. What does no antibiotic mean here?

C. That particular treatment is not recommended

The recommendation concerns this treatment in this assessed visit, not all care or future illness.

4. Which closing is most useful?

B. Confirm the individualized follow-up and warning-sign plan.

A clear follow-up and safety-net plan supports care without promising that circumstances cannot change.

05 Conversation answers

Responding to an antibiotic request

E Dialogue gaps

Pages 46-47

- 1. treatment expectation.** Turn 2. Treatment expectation describes what Jules anticipated based on a previous experience, not an indication established today.
- 2. expected benefit.** Turn 4. Expected benefit concerns what a treatment may actually achieve, which cannot be proved by timing alone.
- 3. symptom relief.** Turn 6. Symptom relief means reducing discomfort and remains a care goal even when an antibiotic is not indicated.
- 4. treatment indication.** Turn 8. A treatment indication is a clinical reason supporting a particular treatment, not simply a request or past timing.
- 5. potential harm.** Turn 10. Potential harm means possible adverse consequences that belong in a balanced treatment explanation.
- 6. Antimicrobial resistance.** Turn 12. Antimicrobial resistance concerns microbes withstanding medicines, not a person's body becoming resistant to care.
- 7. contingency plan.** Turn 14. A contingency plan describes what to do if the situation changes rather than assuming the initial course will continue.
- 8. return precautions.** Turn 16. Return precautions identify changes requiring further medical attention and must be provided for the actual clinical situation.
- 9. Reassessment.** Turn 18. Reassessment is a further clinical evaluation when time or changed circumstances makes it appropriate.
- 10. antibiotic stewardship.** Turn 20. Antibiotic stewardship supports appropriate use while retaining attention to benefit, harm, and the patient's care needs.

F Transfer conversation

Page 48

1. relative

Noor: "My relative received a scan."

The request is influenced by a relative's experience, not a supplied scan indication.

2. today

Dr Hale: "Your assessment does not support that scan today."

The recommendation belongs to today's assessed encounter, not every future situation.

3. unavailable

Noor: "The other person's clinical details are unavailable."

The scenario does not supply the relative's history or reasons for imaging.

4. follow-up

Dr Hale: "We still need clear follow-up."

Not recommending the requested scan does not remove the need for appropriate follow-up.

05

06 Briefing and language checks

Discussing preventive screening choices

A Briefing checks / page 49

1. What prompted the discussion?

C. A screening invitation

The supplied document is an invitation rather than a result or diagnosis.

2. What does Dana want explained?

A. Benefits, false alarms, and subsequent testing

Dana asks about the tradeoffs and next steps before deciding.

3. What is not supplied?

D. An individual screening schedule

The exercise does not establish eligibility, an interval, or an individual clinical recommendation.

D Language checks / page 54

1. What does the invitation establish?

A. An opportunity to discuss screening

The invitation itself does not establish disease, results, or universal eligibility.

2. Which explanation of a positive screen is safest?

D. It may require further evaluation; it is not automatically a diagnosis.

Screening results require appropriate interpretation and may lead to diagnostic evaluation.

3. Which risk statement is incomplete?

A. The risk is lower, without an outcome or time period.

A meaningful risk comparison requires the event, population, and time period being compared.

4. What supports informed choice?

A. Explain benefits, limitations, and next steps, then elicit priorities.

An informed choice requires relevant information and attention to the person's priorities.

06 Conversation answers

Discussing preventive screening choices

E Dialogue gaps

Pages 55-56

- 1. screening.** Turn 2. Screening seeks possible disease or risk and an invitation does not itself establish a diagnosis.
- 2. asymptomatic.** Turn 4. Asymptomatic means without the relevant symptoms, which is different from proving that no condition is present.
- 3. Diagnostic testing.** Turn 6. Diagnostic testing seeks to establish or explain a suspected clinical condition rather than merely inviting a screening discussion.
- 4. positive screen.** Turn 8. A positive screen indicates a possible concern requiring evaluation and is not automatically a confirmed diagnosis.
- 5. false alarm.** Turn 10. A false alarm suggests a problem that subsequent investigation does not confirm, one possible limitation to discuss.
- 6. overdiagnosis.** Turn 12. Overdiagnosis is detecting a condition that would not have caused harm during the person's lifetime, distinct from a false alarm.
- 7. Absolute risk.** Turn 14. Absolute risk describes the chance of an outcome over a defined period, rather than only a ratio between groups.
- 8. eligibility.** Turn 16. Eligibility means meeting the criteria for the relevant option and is not established solely by reading a general invitation.
- 9. shared decision-making.** Turn 18. Shared decision-making combines relevant evidence with the person's priorities rather than substituting pressure for discussion.
- 10. informed choice.** Turn 20. An informed choice follows an adequate explanation of options and consequences and respects the person's decision.

F Transfer conversation

Page 57

1. population

Kim: "The leaflet describes a study population."

The figure concerns a defined study group, not a guaranteed individual outcome.

2. years

Dr Reed: "Its time horizon is five years."

Five years is the explicit period attached to the comparison.

3. prediction

Kim: "It is not a guaranteed personal prediction."

A population statistic does not determine exactly what will happen to Kim.

4. context

Dr Reed: "Individual interpretation needs clinical context."

The scenario states that personal interpretation requires the relevant clinical circumstances.

07 Briefing and language checks

Opening a mental-health conversation

A Briefing checks / page 58

1. What impact does Chris report?

D. Mood and sleep difficulties affecting work

The case describes reported difficulties and their effect, not a completed diagnosis.

2. What remains incomplete?

A. The risk assessment

No risk assessment or risk level has been established in the brief.

3. What must not be promised?

A. Absolute secrecy

Confidentiality has limits that depend on the actual situation and applicable requirements.

D Language checks / page 63

1. Which safety question is direct and neutral?

A. Have you had thoughts about ending your life?

The direct question invites an honest answer without shame or an assumed conclusion.

2. Which privacy statement is inappropriate?

A. I promise absolute secrecy in every circumstance.

Absolute secrecy may conflict with applicable safety and legal duties.

3. What does a warm handoff add?

D. A supported introduction with relevant context

A warm handoff connects care while preserving appropriate consent and information boundaries.

4. Which inference must be avoided?

B. A calm appearance proves low risk.

Appearance alone cannot establish a person's safety or replace a risk assessment.

07 Conversation answers

Opening a mental-health conversation

E Dialogue gaps

Pages 64-65

- 1. low mood.** Turn 2. Low mood describes Chris's reported emotional difficulty without itself establishing a specific diagnosis.
- 2. anhedonia.** Turn 4. Anhedonia refers to reduced interest or pleasure, which can be explored without diagnosing from the word alone.
- 3. daily functioning.** Turn 6. Daily functioning concerns ordinary activities such as work and self-care, not just how someone appears in a consultation.
- 4. suicidal thoughts.** Turn 8. Suicidal thoughts concern ending one's life and require direct, sensitive clinical assessment rather than assumptions.
- 5. confidentiality limits.** Turn 10. Confidentiality limits are circumstances where applicable duties prevent a promise of absolute secrecy.
- 6. nonjudgmental language.** Turn 12. Nonjudgmental language explores the person's experience without assigning blame or shaming a disclosure.
- 7. support network.** Turn 14. A support network consists of people or services that can help, while their involvement still needs appropriate discussion.
- 8. warm handoff.** Turn 16. A warm handoff provides a supported introduction with context rather than leaving the patient with only a contact number.
- 9. consent to share.** Turn 18. Consent to share concerns permission for specified information, within the applicable safety and legal requirements.
- 10. risk assessment.** Turn 20. The risk assessment is still required and cannot be replaced by a calm appearance or a promised referral.

F Transfer conversation

Page 66

1. Jo

Rae: "I agree to meet counselor Jo."

Jo is the counselor named for the supported introduction.

2. share

Dr Moss: "We will clarify the information to share."

Agreement to an introduction does not by itself settle every disclosure detail.

3. complete

Rae: "The safety assessment is not complete."

The scenario explicitly states that the safety assessment remains unfinished.

4. established

Dr Moss: "No risk level has been established."

The supplied facts do not support assigning a clinical risk category.

08 Briefing and language checks

Closing the referral loop

A Briefing checks / page 67

1. What has the specialist office confirmed?

A. Receipt of the referral

Only receipt is confirmed; the later stages have not been established.

2. Who is checking the status?

B. Mina

Mina is the practice coordinator assigned to check and update the status.

3. What does Friday mean?

D. A promised status update

Friday refers to communication about progress and is not a clinical waiting instruction.

D Language checks / page 72

1. Which update is supported?

A. Receipt is confirmed; an appointment is not.

Only receipt has been verified in the supplied referral history.

2. Which phrase could dangerously confuse two processes?

D. Wait until Friday with any new symptoms.

A routine administrative update cannot determine how long a new clinical concern can wait.

3. What makes the next step accountable?

D. A named owner and a defined update

Naming Mina and the update date makes the communication task explicit and trackable.

4. What remains important after sending a referral?

B. Following the response and clarifying continuing responsibilities

Sending a request is only one step in a connected referral process.

08 Conversation answers

Closing the referral loop

E Dialogue gaps

Pages 73-74

- 1. referral receipt.** Turn 2. Referral receipt confirms that the service obtained the request, not that an appointment has been arranged.
- 2. Appointment confirmation.** Turn 4. Appointment confirmation establishes a specific booking and is a later stage than merely receiving the referral.
- 3. consultation question.** Turn 6. The consultation question states the unresolved issue the referring clinician asks the specialist to address.
- 4. record transfer.** Turn 8. Record transfer sends relevant information through the proper process so the receiving team has the necessary context.
- 5. pending action.** Turn 10. A pending action is a task still to complete; here it is checking progress, not attending a booked consultation.
- 6. interim care.** Turn 12. Interim care covers appropriate care while another assessment is pending and cannot be assumed to belong to nobody.
- 7. escalation route.** Turn 14. The escalation route connects new or worsening clinical concerns to appropriate assessment, separate from routine referral tracking.
- 8. contact preference.** Turn 16. Contact preference identifies an appropriate communication method and helps an update reach the patient without avoidable privacy problems.
- 9. consultant letter.** Turn 18. A consultant letter communicates the specialist's assessment or advice back to the referring clinician for review.
- 10. closed-loop referral.** Turn 20. A closed-loop referral tracks the request, response, and returned information instead of stopping at the act of sending.

F Transfer conversation

Page 75

1. Monday

Clinician: "The consultation occurred on Monday."

Monday is the completed attendance date stated in this follow-up case.

2. arrived

Zoe: "The report has not arrived."

Attendance does not mean the consultant's report has reached the referring team.

3. Wednesday

Clinician: "The update is due Wednesday."

Wednesday is the communication commitment, not a treatment deadline.

4. supplied

Zoe: "No new treatment instruction has been supplied."

The exercise provides no new clinical instruction to infer or act on.

08

09 Conversation answers

A consultation through a qualified interpreter

E Extended conversation

Pages 76-77

- 1. direct address.** Turn 3. Direct address keeps the patient as the person being consulted rather than turning the interpreter into the recipient of clinical questions.
- 2. clarification.** Turn 7. Clarification resolves an unclear term or message before it is interpreted as though its meaning were already certain.
- 3. qualified interpretation.** Turn 11. Qualified interpretation provides trained language support and does not transfer clinical decision-making or consent from the patient to the interpreter.
- 4. teach-back.** Turn 15. Teach-back checks how the explanation was understood and should be directed to the patient through the interpreter rather than answered by the interpreter.
- 5. source attribution.** Turn 18. Source attribution identifies who supplied the information and prevents an interpreted patient account from being confused with the interpreter's own opinion.
- 6. closing summary.** Turn 20. A closing summary gathers the agreed information and next step so the patient can confirm the meaning through the same language support.

F Transfer exchange

Page 78

1. idea

Interpreter: "Please give one idea at a time."

The request is for manageable segments rather than a long uninterrupted explanation.

2. pause

Clinician: "I will pause between them."

Pausing allows each segment to be interpreted before more information is added.

3. meaning

Interpreter: "I will preserve the full meaning."

The interpreter aims to retain the content rather than replace it with a summary.

4. decision-maker

Clinician: "The patient remains the decision-maker."

Interpretation supports the patient's participation and does not transfer their decision-making role.

10 Conversation answers

A work note without unnecessary disclosure

E Extended conversation

Pages 79-80

- 1. functional information.** Turn 4. Functional information describes relevant abilities or limitations and is different from disclosing an entire diagnostic or personal history.
- 2. job demands.** Turn 8. Job demands are the actual tasks and conditions relevant to assessing work-related function rather than a generic assumption about an occupation.
- 3. disclosure scope.** Turn 12. Disclosure scope defines the information, purpose, and recipient covered by the relevant authorization rather than treating one request as unlimited permission.
- 4. occupational recommendation.** Turn 16. An occupational recommendation concerns work-related advice based on appropriate assessment and is not a guarantee that an employer will adopt a particular arrangement.
- 5. evidence limitation.** Turn 18. An evidence limitation identifies what the assessment cannot support and prevents a form's wording from forcing an invented clinical conclusion.
- 6. authorized recipient.** Turn 20. An authorized recipient is the person or service permitted to receive the specified information through the applicable process.

F Transfer exchange

Page 81

1. unrelated

Patient: "Those past diagnoses are unrelated to the stated purpose."

The scenario identifies the requested history as unrelated to the work-note purpose.

2. authorized

Clinician: "Disclosure has not been authorized."

The patient has not authorized sharing the additional historical information.

3. requirements

Patient: "Please clarify the actual requirements."

The appropriate process must establish what information is actually required.

4. permission

Clinician: "We will not assume unlimited permission."

A request for a work note does not itself authorize every possible disclosure.

11 Conversation answers

Closing an unanswered result call

E Extended conversation

Pages 82-83

- 1. contact permission.** Turn 4. Contact permission specifies what communication is authorized through a particular route and should be checked before sensitive details are left.
- 2. unsuccessful contact attempt.** Turn 6. An unsuccessful contact attempt records that contact was tried but not achieved, preventing it from being mistaken for a completed clinical explanation.
- 3. clinical priority.** Turn 10. Clinical priority is determined from the actual result and circumstances and cannot be inferred from an unanswered telephone call.
- 4. follow-up ownership.** Turn 12. Follow-up ownership identifies who must complete the next action so an unanswered contact does not disappear between team members.
- 5. acknowledged escalation.** Turn 16. An acknowledged escalation includes confirmation that the relevant person received and accepted the concern rather than merely having a message sent.
- 6. closed-loop follow-up.** Turn 20. Closed-loop follow-up verifies that the relevant communication and responsibility were completed or appropriately escalated, not simply attempted.

F Transfer exchange

Page 84

- 1. sent**
Coordinator: "The escalation was sent."
The coordinator has transmitted the escalation to the appropriate clinician.
- 2. acknowledged**
Clinician: "Receipt has not been acknowledged."
No acknowledgment has arrived, so acceptance cannot be assumed.
- 3. open**
Coordinator: "The task remains open."
The scenario explicitly keeps the task open pending the appropriate process.
- 4. process**
Clinician: "We must follow the actual service process."
The response follows the real clinical and operational arrangements rather than an invented waiting rule.

Find the move you need

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

01 Agreeing on a consultation agenda

Open the agenda: What else were you hoping we could cover?

Avoid dismissal: I am not setting it aside because it was not on the booking.

Check priorities: Have I captured the concerns in the right order?

Full phrase set: pages 7-8 | Dialogue: pages 10-11

02 Taking a precise symptom history

Clarify a label: What does dizzy feel like for you?

Separate two times: Is that the length of one episode or the time since it began?

Summarize carefully: I have the sequence; I have not established the cause.

Full phrase set: pages 16-17 | Dialogue: pages 19-20

03 Reconciling medicines without blame

Ask about actual use: Walk me through what you actually take.

Avoid premature correction: I need to verify the current instructions first.

Agree on verification: We will check the source before finalizing the list.

Full phrase set: pages 25-26 | Dialogue: pages 28-29

04 Explaining an uncertain test result

Acknowledge the worry: Seeing a flag before an explanation can be unsettling.

Bring in context: I will compare it with your symptoms and previous results.

Check the meaning: What does the flag mean to you after that explanation?

Full phrase set: pages 34-35 | Dialogue: pages 37-38

Keep the conversation productive

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

05 Responding to an antibiotic request

Acknowledge experience: I can see why your previous experience shaped your expectation.

Keep the need visible: Your need for relief still matters.

Check the contingency: Let us confirm what changes mean you should seek further help.

Full phrase set: pages 43-44 | Dialogue: pages 46-47

06 Discussing preventive screening choices

Explain the invitation: This invites a discussion; it does not announce a diagnosis.

Name the tradeoff: We should discuss possible benefit as well as false alarms.

Review eligibility: We need to check which options fit your circumstances.

Full phrase set: pages 52-53 | Dialogue: pages 55-56

07 Opening a mental-health conversation

Open sensitively: How have things been emotionally as well as physically?

Avoid an assumption: I cannot judge that from how you look today.

Keep the person involved: I will explain the next steps as clearly as I can.

Full phrase set: pages 61-62 | Dialogue: pages 64-65

08 Closing the referral loop

State the confirmed stage: The receiving office has confirmed receipt.

Define the date: Friday is our update date, not your appointment date.

Close the loop: We will track the response and the information returned after consultation.

Full phrase set: pages 70-71 | Dialogue: pages 73-74

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

absolute risk	explain absolute risk	50
access barrier	identify an access barrier	69
acknowledgment	obtain an acknowledgment	68
active ingredient	compare active ingredients	23
acute concern	assess an acute concern	5
adherence	discuss barriers to adherence	23
adverse reaction	describe an adverse reaction	23
anhedonia	ask about anhedonia	59
antibiotic	discuss an antibiotic recommendation	41
antibiotic stewardship	support antibiotic stewardship	41
antimicrobial resistance	reduce antimicrobial resistance	41
appointment confirmation	obtain appointment confirmation	68
associated symptom	ask about associated symptoms	14
asymptomatic	describe an asymptomatic population	50
average risk	assess average-risk eligibility	50
awaiting review	mark a referral as awaiting review	68
bacterial infection	assess a suspected bacterial infection	41
baseline	compare with the baseline	32
behavioral health	coordinate behavioral-health care	59
brand name	record the brand name	23
care coordination	improve care coordination	6
care transition	support a care transition	69
character	clarify symptom character	15
chronology	preserve the chronology	15

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

clinical handoff confirm the clinical handoff	6
clinical handoff confirm the clinical handoff	15
clinical handoff confirm the clinical handoff	24
clinical handoff confirm the clinical handoff	33
clinical handoff confirm the clinical handoff	42
clinical handoff confirm the clinical handoff	51
clinical handoff confirm the clinical handoff	60
clinical handoff confirm the clinical handoff	69
clinical significance assess clinical significance	32
closed-loop referral complete a closed-loop referral	68
comorbidity review relevant comorbidity	5
concern elicitation use open concern elicitation	6
confidentiality limit explain confidentiality limits	59
confirmatory test discuss a confirmatory test	33
consent to share clarify consent to share	60
consultant letter review the consultant letter	68
consultation agenda agree on the consultation agenda	5
consultation question state the consultation question	68
contact preference verify the contact preference	69
contingency plan agree on a contingency plan	42
continuity of care maintain continuity of care	5
crisis response activate the appropriate crisis response	60
daily functioning assess daily functioning	60
decision aid use a decision aid	51

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

diagnostic testing	distinguish diagnostic testing	50
diagnostic threshold	clarify a diagnostic threshold	42
diagnostic uncertainty	communicate diagnostic uncertainty	15
differential diagnosis	discuss the differential diagnosis	32
discontinued medicine	verify a discontinued medicine	24
distress	acknowledge distress	60
documented plan	review the documented plan	6
documented plan	review the documented plan	15
documented plan	review the documented plan	24
documented plan	review the documented plan	33
documented plan	review the documented plan	42
documented plan	review the documented plan	51
documented plan	review the documented plan	60
documented plan	review the documented plan	69
dosage form	verify the dosage form	24
drug allergy	clarify a reported drug allergy	23
duplicate therapy	check for duplicate therapy	23
duration	clarify episode duration	14
eligibility	review screening eligibility	50
episodic	describe episodic symptoms	14
escalation route	use the clinical escalation route	69
examination finding	explain an examination finding	42
expected benefit	explain the expected benefit	42
false alarm	explain a possible false alarm	50

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

false-negative result explain a false-negative result	32
false-positive result explain a false-positive result	32
family history update the family history	51
follow-up test explain a follow-up test	51
follow-up visit arrange a follow-up visit	5
frequency describe symptom frequency	14
functional limitation describe a functional limitation	5
generic name confirm the generic name	23
history of present illness summarize the history of present illness	15
incidental finding explain an incidental finding	32
informed choice support an informed choice	51
interaction assess a possible interaction	23
interim care clarify interim care	69
intolerance distinguish intolerance from allergy	23
laboratory result interpret a laboratory result	32
location specify the symptom location	14
long-term condition review a long-term condition	5
low mood explore low mood	59
medication reconciliation perform medication reconciliation	23
medication review arrange a medication review	24
medicine list verify the medicine list	23
missed appointment follow up a missed appointment	68
multimorbidity manage multimorbidity	5
nonjudgmental language use nonjudgmental language	59

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

onset establish the reported onset	14
out-of-range flag review an out-of-range flag	32
over-the-counter product disclose over-the-counter products	23
overdiagnosis discuss overdiagnosis	50
patient preference elicit the patient preference	6
patient preference elicit the patient preference	15
patient preference elicit the patient preference	24
patient preference elicit the patient preference	33
patient preference elicit the patient preference	42
patient preference elicit the patient preference	51
patient preference elicit the patient preference	60
patient preference elicit the patient preference	69
patient-centered care provide patient-centered care	5
pending action identify a pending action	68
pending result track a pending result	33
persistent report persistent discomfort	14
pertinent negative document a pertinent negative	14
positive screen explain a positive screen	50
potential harm discuss potential harm	42
preanalytical variation review preanalytical variation	32
predictive value discuss predictive value	33
prescriber contact the original prescriber	24
presenting concern elicit the presenting concern	5
prevention discuss preventive care	50

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

priority establish the clinical priority	5
problem list update the problem list	5
progression describe symptom progression	14
protective factor explore protective factors	59
psychosocial history take a psychosocial history	59
qualified interpreter request a qualified interpreter	6
qualified interpreter request a qualified interpreter	15
qualified interpreter request a qualified interpreter	24
qualified interpreter request a qualified interpreter	33
qualified interpreter request a qualified interpreter	42
qualified interpreter request a qualified interpreter	51
qualified interpreter request a qualified interpreter	60
qualified interpreter request a qualified interpreter	69
radiation describe pain radiation	14
reassessment arrange clinical reassessment	42
record discrepancy resolve a record discrepancy	24
record transfer verify record transfer	69
reference interval explain the reference interval	32
referral send a specialist referral	68
referral receipt confirm referral receipt	68
relative risk distinguish relative risk	50
relieving factor ask about relieving factors	14
repeat sample request a repeat sample	32
results ownership clarify results ownership	68

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

return precautions	explain return precautions	41
review of systems	conduct a focused review of systems	15
risk assessment	complete a risk assessment	59
risk factor	identify a relevant risk factor	51
route of administration	confirm the route of administration	24
safety-netting	provide specific safety-netting	6
screening	discuss screening options	50
screening interval	review the screening interval	51
self-limiting illness	explain a self-limiting illness	41
sensitivity	explain test sensitivity	33
severity	assess symptom severity	15
shared agenda	negotiate a shared agenda	6
shared decision-making	support shared decision-making	50
signposting	use clear signposting	6
sleep disturbance	describe sleep disturbance	59
source attribution	preserve source attribution	6
source attribution	preserve source attribution	15
source attribution	preserve source attribution	24
source attribution	preserve source attribution	33
source attribution	preserve source attribution	42
source attribution	preserve source attribution	51
source attribution	preserve source attribution	60
source attribution	preserve source attribution	69
specificity	explain test specificity	33

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

stressor identify a current stressor	59
suicidal thoughts ask directly about suicidal thoughts	59
support network identify a support network	60
symptom relief discuss symptom relief	41
teach-back use teach-back without blame	6
teach-back use teach-back without blame	15
teach-back use teach-back without blame	24
teach-back use teach-back without blame	33
teach-back use teach-back without blame	42
teach-back use teach-back without blame	51
teach-back use teach-back without blame	60
teach-back use teach-back without blame	69
therapeutic alliance strengthen the therapeutic alliance	60
therapeutic relationship maintain the therapeutic relationship	41
treatment expectation explore the treatment expectation	41
treatment indication establish a treatment indication	41
trend examine the result trend	32
trigger identify a reported trigger	14
urgency category verify the urgency category	68
viral illness explain a viral illness	41
visit length explain the visit length	6
warm handoff arrange a warm handoff	59
watchful waiting explain watchful waiting	41
working diagnosis qualify a working diagnosis	33

Original cases. Clear boundaries.

These are original fictional teaching conversations, not transcripts of real workers. Names, figures, organizations, and incidents are invented. Sources support terminology and communication aims, not the fictional results or an endorsement of this book.

[1] AHRQ. Use the Teach-Back Method: Tool 5.

[Open the original source](#)

Supports checking the clarity of an explanation without treating the patient as the problem. The original conversations are language practice, not copied clinical scripts. Checked 10 October 2026.

[2] AAFP. Continuity of Care.

[Open the original source](#)

Context for sustained primary-care relationships and coordination across encounters. Checked 10 October 2026.

[3] CDC. Healthy Habits: Antibiotic Do's and Don'ts.

[Open the original source](#)

Background for antibiotic communication; fictional cases do not establish prescribing rules. Checked 10 October 2026.

[4] AHRQ. Make Referrals Easy: Tool 21.

[Open the original source](#)

Context for referral communication and follow-up responsibilities. Checked 10 October 2026.

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